

Access guide — ulms-highgrade-pulm-mets-tp53-smarc b1-hla-a2-w9t4

How to access each therapy: trial contacts + manufacturer medical-info

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How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a [Verified](#) date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	Documented ECOG performance status	diagnostic	Standard of care	Not applicable; a clinical assessment.	Have the treating oncologist record an ECOG score at the next clinic visit and put it in the note, not in a verbal handoff.
2	Pazopanib	small_molecule	Standard of care + Off-label use	FDA-approved for advanced soft tissue sarcoma in patients who have received prior chemotherapy.	Hold until after a chemotherapy line; the label's prior-chemotherapy requirement is the only gate.
3	Pulmonary metastasectomy	surgery	Standard of care	Not a drug; standard surgical practice.	Get a thoracic surgical opinion on the current imaging now, not after induction. Whether she is ever a resection candidate changes how the systemic plan is framed.
4	SBRT to pulmonary metastases	radiotherapy	Standard of care	Not a drug; standard radiotherapy practice.	Raise with radiation oncology after systemic control is established and the number of active lesions is clear.
5	Tissue order A: one comprehensive tumour DNA + RNA profile	diagnostic	Standard of care	Comprehensive genomic profiling on FFPE is routine clinical testing; several of the assays carry FDA approval as companion diagnostics.	Request release of the 2024 uterine block and the 2026 pulmonary core from the outside institution today. This is the slow step, it is administrative rather than clinical, and it sits in front of every other test in this dossier.
6	Tissue order B: one IHC block cut (INI1, MMR, p53, Rb, ATRX, PD-L1, B7-H3)	diagnostic	Standard of care	Routine diagnostic PD-L1 assays carry status for specific agents.	Order all seven stains on one requisition against the same block cut. Serial ordering is the failure mode here.

7	Tissue order C: cancer/testis antigen IHC set (MAGE-A4, PRAME, NY-ESO-1)	diagnostic	Standard of care	Local IHC is routine; the companion diagnostics used by TCR-T programs are separate FDA-approved or cleared assays run centrally.	Order all three on one block cut. NY-ESO-1 in particular should not be ordered alone; if the shared cut is not happening, this stain should not happen either.
8	Trabectedin monotherapy	small_molecule	Standard of care + Off-label use	FDA-approved (Yondelis, 2015) for unresectable or metastatic liposarcoma or leiomyosarcoma after a prior regimen.	No separate access action beyond the LMS-04 prior authorization already described.
9	Abemaciclib + gemcitabine and docetaxel	small_molecule	Off-label use + Clinical trial	Abemaciclib (Verzenio) FDA-approved in HR-positive HER2-negative breast cancer. No sarcoma indication. Gemcitabine and docetaxel are generic.	Call MD Anderson at 866-632-6789 about NCT06498648 phase 2 and ask directly whether the treatment-naïve stratum is currently open and whether an ECOG of 2 would still screen.
10	Abemaciclib + gemcitabine and docetaxel	small_molecule	Off-label use + Clinical trial	Abemaciclib (Verzenio) FDA-approved in HR-positive HER2-negative breast cancer. No sarcoma indication. Gemcitabine and docetaxel are generic.	Order Rb by IHC on the tissue-order-B block now. One stain routes her between this trial (Rb intact, first-line eligible today) and REC-617 (Rb lost, after one prior line). The block cut was already priced; this adds no tissue cost, only the instruction to read the result against both protocols.

11	Afamitresgene autoleucel (MAGE-A4 TCR-T, TECELRA)	cell_therapy	Off-label use + Not yet accessible	FDA-approved (TECELRA) for adults and patients 12 and older with unresectable or metastatic synovial sarcoma who have received prior chemotherapy, are positive, and whose tumour expresses MAGE-A4 by an FDA-approved or cleared companion diagnostic. Label revised mid-2026 with the accelerated-approval statement removed.	Do not present this as an open option on the strength of the HLA type alone. The allele is necessary and nowhere near sufficient.
12	All-trans retinoic acid + cemiplimab	combination	Off-label use + Clinical trial	ATRA (tretinoin) FDA-approved in acute promyelocytic leukaemia. Cemiplimab (Libtayo) FDA-approved in cutaneous squamous cell carcinoma, basal cell carcinoma and NSCLC. Neither is approved in sarcoma.	Email after the first chemotherapy line to confirm the cohort is still open.
13	Dostarlimab	monoclonal_antibody	Off-label use	FDA-approved for dMMR recurrent or advanced solid tumours that have progressed on prior treatment, and in dMMR/MSI-H endometrial cancer. No sarcoma-specific indication.	Add MMR IHC (MLH1, PMS2, MSH2, MSH6) to tissue order B; it rides the same block cut as INI1 and costs almost nothing extra.
14	Doxorubicin + ifosfamide (intensified)	small_molecule	Off-label use	Both FDA-approved, generic; no uterine leiomyosarcoma indication on either label.	Same baseline cardiac and pulmonary workup as any anthracycline plan.

15	Doxorubicin + pembrolizumab (first-line)	combination	Off-label use	Doxorubicin generic; pembrolizumab FDA-approved in other indications. The combination has no sarcoma indication.	Not a realistic first-line substitution for LMS-04 on access grounds.
16	Doxorubicin + trabectedin induction, then trabectedin maintenance (LMS-04)	small_molecule	Off-label use	Doxorubicin: FDA-approved, generic. Trabectedin (Yondelis): FDA-approved 2015 for unresectable or metastatic liposarcoma or leiomyosarcoma after a prior regimen. The first-line combination sits outside that label.	Document an ECOG score at the next clinic visit. It is undocumented, it gates most of the trial options in this dossier, and it costs nothing.
17	Eribulin mesylate	small_molecule	Off-label use	FDA-approved for unresectable or metastatic liposarcoma after a prior regimen, and for metastatic breast cancer. Not approved for leiomyosarcoma.	Defer. Revisit after anthracycline and gemcitabine-based lines.
18	Gemcitabine + dacarbazine	small_molecule	Off-label use	Both FDA-approved, generic; no sarcoma indication on either label.	No action now. Revisit at progression.
19	Gemcitabine + docetaxel	small_molecule	Off-label use + Clinical trial	Both FDA-approved for other indications and available as generics; neither carries a sarcoma indication.	Hold as the backup plan pending the LVEF and PFT results that gate doxorubicin.
20	Gemcitabine + docetaxel +/- bevacizumab (GOG-0250)	combination	Off-label use	Gemcitabine and docetaxel generic; bevacizumab FDA-approved in several solid tumours with multiple biosimilars. No uterine leiomyosarcoma indication.	Use gemcitabine and docetaxel without bevacizumab if this backbone is chosen.

21	Gemcitabine + nab-sirolimus	small_molecule	Off-label use + Clinical trial + Expanded access program	Nab-sirolimus (FYARRO) FDA-approved for locally advanced unresectable or metastatic malignant PEComa. Gemcitabine generic. The combination is investigational.	Include TSC1 and TSC2 in tissue order A. It is a standard part of comprehensive panels and costs nothing extra to report.
22	Larotrectinib (TRK inhibition, NTRK-fusion gated)	small_molecule	Off-label use + Expanded access program	FDA-approved (Vitrakvi) for adult and paediatric solid tumours with an NTRK gene fusion without a known acquired resistance mutation, that are metastatic or where surgical resection is likely to cause severe morbidity, and that have no satisfactory alternative treatment or have progressed following treatment.	Include RNA-based fusion calling in tissue order A. Pan-TRK IHC is a screen only and a positive would still need fusion-capable confirmation, which costs an extra two weeks.
23	Lurbinectedin + doxorubicin	small_molecule	Off-label use	Lurbinectedin (Zepzelca) FDA-approved for metastatic small cell lung cancer. No sarcoma indication.	Confirm with the SaLuDo sites that accrual is truly closed rather than paused before writing it off; the registry status is 'active, not recruiting'.
24	Nivolumab	monoclonal_antibody	Off-label use	FDA-approved across multiple solid tumours; no sarcoma indication.	No action. Recorded so the negative is visible rather than absent.
25	Nivolumab +/- ipilimumab	monoclonal_antibody	Off-label use	Both FDA-approved across multiple solid tumours; no sarcoma indication.	Defer to later lines.
26	Olaparib + temozolomide	small_molecule	Off-label use	Olaparib (Lynparza) FDA-approved in BRCA-mutated ovarian, breast, pancreatic and prostate cancers. Temozolomide approved in glioma. Neither is approved in sarcoma.	Send tissue order A (DNA plus RNA comprehensive profiling with an HRD or genomic-instability score) before anything else; it is the gate on this whole branch.

27	PARP inhibition in BRCA2-altered uterine leiomyosarcoma	small_molecule	Off-label use	PARP inhibitors approved across BRCA-mutated ovarian, breast, pancreatic and prostate cancers. No sarcoma indication.	Order tissue HRR sequencing with a genomic-instability score as part of tissue order A.
28	Pegylated liposomal doxorubicin (maintenance)	small_molecule	Off-label use + Not yet accessible	FDA-approved (Doxil) for ovarian cancer after platinum, AIDS-related Kaposi sarcoma, and multiple myeloma. No soft tissue sarcoma indication.	Park until the maintenance phase of LMS-04 is underway and the question is live.
29	Pembrolizumab	monoclonal_antibody	Off-label use	FDA-approved across many tumour types, including tumour-agnostic indications for MSI-high/dMMR and for TMB-high (≥ 10 mut/Mb) solid tumours after prior therapy. No sarcoma-specific indication.	Include validated tissue TMB and MSI reporting in tissue order A, and MMR IHC in tissue order B. Both are cheap and both can open an on-label door.
30	Tazemetostat	small_molecule	Off-label use	FDA-approved (accelerated) for metastatic or locally advanced epithelioid sarcoma not eligible for complete resection in patients 16 and older, and for relapsed/refractory follicular lymphoma. Not approved in leiomyosarcoma.	Order INI1/SMARCB1 IHC (BAF47 clone) on tumour tissue as part of tissue order B. That single stain decides whether this branch exists.
31	Tebentafusp (gp100 x CD3 ImmTAC)	bispecific_fusion	Off-label use + Clinical trial	FDA-approved (KIMMTRAK) for adults with unresectable or metastatic uveal melanoma. No sarcoma indication.	No realistic action. Included because the ImmTAC class precedent is cited in the dossier and a reader will ask whether the approved member is reachable.
32	Temozolomide	small_molecule	Off-label use	FDA-approved for glioma indications; generic. No sarcoma indication.	Treat as an add-on to the olaparib decision rather than a separate access question.

33	Tiragolumab + atezolizumab	monoclonal_antibody	Off-label use	Atezolizumab (Tecentriq) FDA-approved in several solid tumours including alveolar soft part sarcoma. Tiragolumab investigational, no approval.	Ask the NCI trial referral line whether NCT05286801 has any plan to reopen; a 40-site study that closed to accrual sometimes adds cohorts.
34	Tislelizumab + liposomal doxorubicin and ifosfamide	combination	Off-label use + Not yet accessible	Tislelizumab (Tevimbra) FDA-approved in oesophageal squamous cell and junction adenocarcinoma. No sarcoma indication.	No US access to the trial. Confirm before writing off, but the registry lists one Chinese site.
35	ADCE-D01 (antibody-drug conjugate)	antibody_drug_conjugate	Clinical trial	Investigational; no approval anywhere.	Email rene.smrcka@adcendo.com now to ask which US sites are screening and what the current slot situation is; the answer will still be useful in six months.
36	ATR inhibition (ATRX / ALT-directed hypothesis)	small_molecule	Clinical trial	No ATR inhibitor is approved in any indication.	Include ATRX in tissue order A and, if available, ATRX loss by IHC in tissue order B. That is the cheap part and it informs biology regardless.
37	Anlotinib (catequentinib, AL3818)	small_molecule	Clinical trial + Not yet accessible	Approved in China (NMPA) for several indications including soft tissue sarcoma. No FDA or EMA approval.	Email Shiyings@advenchen.com to ask whether the leiomyosarcoma indication has reopened and whether Indication E would accept a leiomyosarcoma patient.
38	B7-H3-directed antibody-drug conjugates (MGC026;	antibody_drug_conjugate	Clinical trial	All investigational; no B7-H3 ADC is approved anywhere.	Add B7-H3 (CD276) IHC to tissue order B. It rides the same block cut as INI1 and MMR and tells her whether the class has a target here at all.
39	Brenetafusp (IMC-F106C, PRAME x CD3 ImmTAC)	bispecific_fusion	Clinical trial	Investigational; no approval. Sponsor markets tebentafusp in the same ImmTAC class.	Call Immunocore at 1-844-466-8661 and ask whether any brenetafusp cohort is reopening or whether a successor PRAME study is registered.
40	CC-3 (B7-H3 x CD3 bispecific antibody)	bispecific_antibody	Clinical trial + Not yet accessible	Investigational; no approval anywhere.	No action. Recorded so the B7-H3 x CD3 branch shows as checked and closed rather than absent.

41	CDR404 (MAGE-A4 x CD3 T-cell engager)	bispecific_antibody	Clinical trial	Investigational; no approval anywhere.	Email and ask whether a systemic-therapy-naive patient can enrol, and which US sites are currently screening.
42	FAP-directed radioligand therapy ([225Ac]RTX-2358, [212Pb]Pb-PSV359,	radioligand	Clinical trial	All investigational; no FAP-directed radioligand is approved anywhere.	Not accessible before induction. The Ratio study requires relapsed and refractory disease and the others expect prior lines.
43	Fluzoparib + camrelizumab with concurrent SBRT	combination	Clinical trial + Not yet accessible	Both approved in China; neither has FDA or EMA approval.	No US access. The transferable idea, SBRT to lung lesions combined with systemic therapy, can be pursued locally without this protocol.
44	IMA203 / IMA203CD8 (PRAME-directed TCR-T)	cell_therapy	Clinical trial	Investigational; no approval anywhere.	Email ctgovinquiries@immatics.com now, before induction, with two questions: does uterine leiomyosarcoma fall inside the current solid-tumour cohorts, and does 'received or not eligible for all available indicated standard of care' allow a patient who has declined first-line therapy.
45	In vivo lung perfusion with doxorubicin, plus metastasectomy	surgery	Clinical trial + Not yet accessible	Doxorubicin is approved; the regional perfusion delivery is investigational.	Email marcelo.cypel@uhn.ca and Jennifer.Lister@uhn.ca now and ask two things: does the study accept US-resident patients, and how is the 450 mg doxorubicin exclusion counted against a planned six-cycle induction.
46	Ivonescimab (PD-1 x VEGF bispecific)	bispecific_antibody	Clinical trial + Not yet accessible	Approved in China (NMPA) for non-small cell lung cancer indications. No FDA or EMA approval; investigational in the United States.	Call Dr D'Angelo's team at 646-888-4159 or email this week and state plainly that she is systemic-therapy naive with LMS-04 planned, and ask whether the decline-first-line pathway is currently open and what documentation of the decline the protocol requires.
47	Letetresgene autoleucel (lete-cel, NY-ESO-1 TCR-T)	cell_therapy	Clinical trial + Not yet accessible	Investigational; no approval anywhere.	Ask USWM at 1-855-246-9232 whether any IGNYTE-ESO cohort is reopening.

48	MASCT-I (autologous dendritic cell and cytokine-induced killer cell therapy) + doxorubicin and ifosfamide	cell_therapy	Clinical trial + Not yet accessible	Investigational; no approval outside the study.	Confirm no US or EU site exists before considering further; as registered there is one site and it is in mainland China.
49	Peposertib (DNA-PK inhibitor) + low-dose pegylated liposomal doxorubicin	small_molecule	Clinical trial	Peposertib investigational; no approval anywhere. Pegylated liposomal doxorubicin FDA-approved in other indications.	Put the 360 mg/m2 arithmetic in front of the treating oncologist before induction starts, not before cycle six: six full cycles at 60 mg/m2 equals exactly 360, so either a five-cycle plan, dose reductions, or a decision to forgo this trial should be explicit up front.
50	Pulmonary suffusion with cisplatin plus metastasectomy	surgery	Clinical trial	Cisplatin is approved; the suffusion delivery is investigational.	Email with the current chest imaging and ask whether her bilateral infiltrative pattern is within the resectable population this study treats.
51	REC-617 (selective CDK7 inhibitor)	small_molecule	Clinical trial	Investigational; no approval anywhere.	Include Rb by IHC in tissue order B so the eligibility question is already answered when the time comes.
52	Tuvusertib (ATR inhibitor) + temozolomide	small_molecule	Clinical trial	Tuvusertib investigational; no approval anywhere. Temozolomide FDA-approved in glioma, generic.	After LMS-04, call the nearest recruiting site (UC Irvine 877-827-8839, Yale 203-785-5702, or Kansas 913-588-3671) and confirm the dose-escalation arm is still open to sarcoma; the phase 2 portion is colorectal-only and the arm structure decides whether a seat exists.
53	Ubamata-mab (MUC16 x CD3 bispecific), alone or with cemiplimab	bispecific_antibody	Clinical trial	Investigational; no approval.	No action. Histology excludes her independently of the enrolment status.
54	Uzatresgene autoleucel (ADP-A2M4CD8), alone or with nivolumab or pembrolizumab	cell_therapy	Clinical trial + Not yet accessible	Investigational; no approval anywhere.	Confirm with USWM at 1-855-246-9232 whether any SURPASS cohort is reopening; the registry status is 'active, not recruiting' rather than terminated.

55	Valemetostat tosylate (EZH1/EZH2 dual inhibitor)	small_molecule	Clinical trial + Not yet accessible	Approved in Japan (Ezharmia) for adult T-cell leukemia/lymphoma and peripheral T-cell lymphoma. No FDA or EMA approval; investigational in solid tumours.	Confirm a US or EU site is accepting international patients before anything else. As registered this is a France-only study and she is US-based.
56	ZL-6201 (LRRC15-directed antibody-drug conjugate)	antibody_drug_conjugate	Clinical trial	Investigational; no approval anywhere.	Email to identify which of the US sites are open and whether LRRC15 expression is required or only collected.
57	Zanzalitinib (XL092)	small_molecule	Clinical trial	Investigational; no approval anywhere.	Not accessible in the first-line window. Park until at least two lines are behind her.
58	Tuvusertib monotherapy in ATRX-mutant astrocytoma (GEINO, Spain)	small_molecule	Not yet accessible	Investigational; no approval anywhere.	No enrolment action. Include ATRX in tissue order A so the relevance of this read-out to her case is decidable.
59	BRD9 degraders (class)	targeted_protein_degrader	Unavailable	No approval anywhere. Both clinical programs terminated.	No action available. Recorded so the branch shows as closed rather than missing.
60	Berzosertib + gemcitabine (randomised phase 2, Institut Bergonié)	small_molecule	Unavailable	Berzosertib investigational; no approval anywhere.	No access action. Ask whoever re-runs the evidence screen to check for the publication at the next progression decision point; a positive read-out would change how the tuvusertib seat is weighed.
61	Berzosertib monotherapy (ATRX-mutant leiomyosarcoma cohort, NCT03718091)	small_molecule	Unavailable	Investigational; no approval anywhere. No active berzosertib monotherapy program in sarcoma.	No action. Carry the negative into any future weighing of ATR seats: monotherapy in her histology has been tried and did not hold.
62	CFT8634 (BRD9 degrader)	targeted_protein_degrader	Unavailable	No approval; phase 1 terminated.	No action available.
63	FHD-609 (BRD9 degrader)	targeted_protein_degrader	Unavailable	No approval; phase 1 terminated.	No action available.
64	with [68Ga]Ga-PSMA-11 PET selection	radioligand	Unavailable	Investigational; the sarcoma application has no approval and no active program.	No action available.

Standard of care (8)

1. Documented ECOG performance status

Aliases: performance status, ECOG PS, Karnofsky, ecog-performance-status-documentation

Modality: diagnostic · **Regulatory:** Not applicable; a clinical assessment. · **Geographic scope:** Wherever she is seen · **Verified:** 2026-08-24

The ECOG of 1 in this record is an intake assumption, never a measured score, and it is the single most widely load-bearing missing item in the dossier. Protocols with an ECOG 0-1 ceiling include the valemestostat, IMA203, CDR404, ADCE-D01, ZL-6201, zanzalitinib, anlotinib and all three FAP radioligand studies. A clinician writing a number in the chart at the next visit is an access action, and it costs nothing.

Next steps

1. Have the treating oncologist record an ECOG score at the next clinic visit and put it in the note, not in a verbal handoff.
2. Record it alongside resting and exertional oximetry taken at her home altitude, with the altitude stated on the result.
3. If the measured score is 2, the reachable set narrows to the ivonescimab study, the abemaciclib study, REC-617, the gemcitabine plus nab-sirolimus study, the ATRA study and both surgical protocols. That is worth knowing before outreach begins, not during it.
4. Symptomatic with cough and dyspnea yet well enough to travel for a specialist opinion is consistent with either 1 or 2. Do not let the assumption stand in for the measurement.

Payer / coverage: No cost. The absence of a documented score is what stops screening, not the score itself.

Notes: Recorded as an access action rather than a clinical detail because that is functionally what it is: an undocumented performance status blocks screening at roughly two thirds of the protocols in this dossier.

2. Pazopanib

Aliases: Votrient, GW786034

Modality: small_molecule · **Regulatory:** FDA-approved for advanced soft tissue sarcoma in patients who have received prior chemotherapy. · **Guidelines:** NCCN soft tissue sarcoma option after prior chemotherapy · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Votrient is approved for advanced soft tissue sarcoma in patients who have had prior chemotherapy, which covers uterine leiomyosarcoma by histology but not her line: she has had no systemic therapy. It becomes on-label the moment induction is behind her. One consequence worth noting now is that taking pazopanib would disqualify her from the ivonescimab trial, which excludes prior VEGF-pathway inhibitors.

Next steps

1. Hold until after a chemotherapy line; the label's prior-chemotherapy requirement is the only gate.

- Before starting any VEGF-pathway drug, decide about NCT07516925 (ivonescimab), which excludes patients with prior VEGF inhibitor exposure.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT00733688		completed	no	—

Manufacturer / sponsor contact

- **Company:** Novartis Pharmaceuticals Corporation
- **Med info phone:** 1-888-669-6682
- **Product info:** <https://www.votrient.com>
- **Notes:** Phone from the current Votrient FDA label.

Payer / coverage: Straightforward once a prior chemotherapy line is documented. Hepatotoxicity monitoring is a label requirement, not a coverage one.

3. Pulmonary metastasectomy

Aliases: metastasectomy, lung resection, IRLM

Modality: surgery · **Regulatory:** Not a drug; standard surgical practice. · **Guidelines:** NCCN soft tissue sarcoma recognises resection of limited pulmonary metastases in selected patients · **Geographic scope:** US-wide at thoracic surgical centres · **Verified:** 2026-08-24

Standard practice at any thoracic surgical centre and covered as such, with the registry data in sarcoma coming from series rather than randomised trials. Her consulting oncologist has already kept local therapy on the table after systemic control. The open question is anatomic, not administrative: bilateral disease described as infiltrative and possibly inflammatory may not be resectable, and a thoracic surgeon should look at the images early rather than after six cycles.

Next steps

- Get a thoracic surgical opinion on the current imaging now, not after induction. Whether she is ever a resection candidate changes how the systemic plan is framed.
- Ask the surgeon explicitly about the infiltrative radiologic pattern and whether it changes resectability.
- Request the September 2024 operative report while arranging the block release; whether morcellation occurred bears on surveillance.

Payer / coverage: Covered as surgical care. Prior authorisation is routine rather than contested.

4. SBRT to pulmonary metastases

Aliases: stereotactic body radiotherapy, SABR

Modality: radiotherapy · **Regulatory:** Not a drug; standard radiotherapy practice. · **Guidelines:** NCCN soft tissue sarcoma recognises SBRT for selected pulmonary metastases · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Available at any radiation oncology department with stereotactic capability and reimbursed as standard care. The sarcoma evidence is single-institution series, which is enough for it to be offered but not enough to displace systemic therapy in multifocal disease. Bilateral disease limits how much can be treated at once.

Next steps

1. Raise with radiation oncology after systemic control is established and the number of active lesions is clear.
2. If considered before then, ask about the interaction with concurrent trabectedin, which has radiosensitising considerations.

Payer / coverage: Routinely covered. Some payers require documentation of a limited number of lesions.

5. Tissue order A: one comprehensive tumour DNA + RNA profile

Aliases: tissue NGS, comprehensive genomic profiling, whole exome plus whole transcriptome, smarcb1-tissue-ngs-concurrent, hrr-somatic-ngs-hrd-score, ntrk-rna-fusion-panel, tmb-tissue-validated-cgp

Modality: diagnostic · **Regulatory:** Comprehensive genomic profiling on FFPE is routine clinical testing; several of the assays carry FDA approval as companion diagnostics. · **Geographic scope:** US-wide; specimens ship to the reference laboratory · **Verified:** 2026-08-24

One requisition on archival tissue that settles TMB, MSI, the homologous-recombination gene set with a genomic-instability score, NTRK and RET fusion calling, RB1, ATRX, MED12, TSC1/TSC2, and tissue confirmation of the plasma TP53 and SMARCB1 calls. It gates the olaparib branch, the tumour-agnostic checkpoint claims, the TRK branch and the CDK7 trial all at once. Ordering these serially instead of together is how a first-line window gets spent.

Next steps

1. Request release of the 2024 uterine block and the 2026 pulmonary core from the outside institution today. This is the slow step, it is administrative rather than clinical, and it sits in front of every other test in this dossier.
2. Send one requisition, not several. Specify DNA plus RNA, TMB and MSI reporting, an HRD or genomic-instability score, fusion calling, and explicit reporting of SMARCB1, TP53, RB1, ATRX, MED12, TSC1 and TSC2.
3. Use the 2026 lung core as the RNA input where possible; RNA quality degrades in blocks older than about five years.
4. Ask the laboratory for expedited handling and tell them the patient is pre-treatment with induction pending. Turnaround is quoted at two to three weeks and that is the whole first-line window.
5. Order the germline homologous-recombination panel in parallel rather than reflexing to it after a somatic finding.

Manufacturer / sponsor contact

- **Company:** Caris Life Sciences (preferred by the target validator); alternatives Tempus AI, Foundation Medicine,

MSK-IMPACT, NeoGenomics

- **Med info phone:** 1-888-979-8669
- **Product info:** <https://www.carislifesciences.com>
- **Notes:** Caris client services 1-888-979-8669 (MI Profile, whole exome plus whole transcriptome). Tempus AI 1-800-739-4137. Foundation Medicine 1-888-988-3639. MSK Diagnostic Molecular Pathology 1-212-639-2000. NeoGenomics 1-866-776-5907. All captured from the target-validation providers list.

Payer / coverage: Comprehensive genomic profiling in metastatic solid tumour is generally covered under the Medicare NGS national coverage determination and by most commercial plans for advanced disease. Most of the reference laboratories run patient-assistance or capped-cost programs; ask at the time of ordering rather than after the bill.

Notes: The block sits at the outside institution that performed the 2024 resection and signed it out benign. Block release requests to a hospital that has since amended its own report can be slow, so put the request in writing, name both specimens, and copy the treating oncologist's office.

6. Tissue order B: one IHC block cut (INI1, MMR, p53, Rb, ATRX, PD-L1, B7-H3)

Aliases: INI1, BAF47, SMARCB1 IHC, MMR IHC, p53, Rb, ATRX, PD-L1, B7-H3, ini1-smarcb1-ihc-tumor, mmr-ihc-msi-tissue, b7h3-cd276-ihc

Modality: diagnostic · **Regulatory:** Routine diagnostic immunohistochemistry; PD-L1 assays carry companion-diagnostic status for specific agents. · **Geographic scope:** US-wide; reference laboratory or the treating institution's own pathology department · **Verified:** 2026-08-24

Seven stains off a single block cut, all of them routine, all of them fast once the block is in hand. INI1 is the one that decides whether the entire SMARCB1 branch exists. MMR opens or closes dostarlimab. Rb decides the CDK7 trial. B7-H3 tells her whether the ADC class has a target. Turnaround is three to five days per the target validator; the delay is the block release, not the laboratory.

Next steps

1. Order all seven stains on one requisition against the same block cut. Serial ordering is the failure mode here.
2. Order INI1 on both the 2024 uterine block and the 2026 pulmonary core. Primary-versus-metastasis discordance is a known problem and a single negative core is not reassuring.
3. Read B7-H3 on the larger 2024 resection rather than the lung core alone; expression is spatially heterogeneous.
4. Ask the pathology laboratory to score INI1 against the internal positive control and to state the control explicitly in the report.
5. Tell the laboratory that induction is pending; three to five days is achievable and worth asking for.

Manufacturer / sponsor contact

- **Company:** NeoGenomics Laboratories (preferred by the target validator); alternatives Mayo Clinic Laboratories, ARUP, Labcorp Oncology, Quest, Discovery Life Sciences
- **Med info phone:** 1-866-776-5907
- **Product info:** <https://neogenomics.com>
- **Notes:** NeoGenomics 1-866-776-5907 runs INI1 on its routine sarcoma menu rather than case by case, which

is the reason the validator preferred it. Mayo Clinic Laboratories 1-800-533-1710. ARUP 1-800-242-2787. Labcorp Oncology 1-800-845-6167. Quest 1-866-697-8378. Discovery Life Sciences 1-800-956-4472.

Payer / coverage: Routine IHC is inexpensive and covered. The cost of ordering these one at a time is measured in weeks, not dollars.

7. Tissue order C: cancer/testis antigen IHC set (MAGE-A4, PRAME, NY-ESO-1)

Aliases: MAGE-A4 IHC, PRAME IHC, NY-ESO-1 IHC, magea4-expression-central, prame-expression-central, nyeso1-expression-central, ct-antigen-target-biology

Modality: diagnostic · **Regulatory:** Local IHC is routine; the companion diagnostics used by TCR-T programs are separate FDA-approved or cleared assays run centrally. · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Three stains off one block cut, and they only make sense together. Her HLA-A*02:01 is confirmed, which means the allele half of TCR-T and ImmTAC eligibility is already done; this is the other half. A local positive is a reason to approach a sponsor, not a substitute for the sponsor's own assay, and every program in this class re-tests on its own device at screening.

Next steps

1. Order all three on one block cut. NY-ESO-1 in particular should not be ordered alone; if the shared cut is not happening, this stain should not happen either.
2. Before ordering, ask the two live programs which assay they accept: ctgovinquiries@immatics.com for PRAME by IMADetect RT-qPCR, and CDR404-001_Study@CDR-Life.com for MAGE-A4. A local stain that the sponsor will not accept still has screening value but should not delay the sponsor's own testing.
3. Do not order these on the strength of the TECELRA approval. That label is synovial sarcoma only and a MAGE-A4 result will not open it.
4. Reserve enough unstained slides that a sponsor's central laboratory can also be supplied without a second block request.

Manufacturer / sponsor contact

- **Company:** NeoGenomics Laboratories (preferred by the target validator); alternatives Mayo Clinic Laboratories, ARUP, Labcorp Oncology, Discovery Life Sciences
- **Med info phone:** 1-866-776-5907
- **Product info:** <https://neogenomics.com>
- **Notes:** NeoGenomics runs MAGE-A4, PRAME and NY-ESO-1 IHC. Mayo Clinic Laboratories 1-800-533-1710. ARUP 1-800-242-2787. Labcorp Oncology 1-800-845-6167. Discovery Life Sciences 1-800-956-4472.

Payer / coverage: Cancer/testis antigen IHC is not always covered as routine care outside a trial context. Ask the laboratory about self-pay pricing before ordering, and ask any sponsor whether it will run its own screening assay at no cost, which several do.

8. Trabectedin monotherapy

Aliases: ET-743, Yondelis, trabectedin-monotherapy

Modality: small_molecule · **Regulatory:** FDA-approved (Yondelis, 2015) for unresectable or metastatic liposarcoma or leiomyosarcoma after a prior anthracycline-containing regimen. · **Guidelines:** NCCN soft tissue sarcoma option for LMS after anthracycline · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Yondelis names leiomyosarcoma on its label, but only after a prior anthracycline-containing regimen. Today she is anthracycline-naive, so single-agent use would be off-label; after LMS-04 induction it becomes on-label, at which point she will already have had it. The practical read is that trabectedin is fully available to her, and the label question is about line, not about the drug.

Next steps

1. No separate access action beyond the LMS-04 prior authorization already described.
2. If the plan changes to single-agent trabectedin first line, the same off-label filing applies.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04323119		recruiting	no	Bruno Vincenzi, Prof/MD; b.vincenzi@unicampus.it; 003906-22541

Manufacturer / sponsor contact

- **Company:** Janssen Products, LP (Johnson & Johnson)
- **Med info phone:** 1-800-526-7736
- **Product info:** <https://www.yondelis.com>

Payer / coverage: On-label after an anthracycline; the appeal work all sits in the first-line combination request, not here.

Off-label use (26)

9. Abemaciclib + gemcitabine and docetaxel

Aliases: Verzenio, LY2835219, abemaciclib-gemcitabine

Modality: small_molecule · **Regulatory:** Abemaciclib (Verzenio) FDA-approved in HR-positive HER2-negative breast cancer. No sarcoma indication. Gemcitabine and docetaxel are generic. · **Geographic scope:** United States; single site, MD Anderson, Houston TX · **Verified:** 2026-08-24

This is the one trial in the dossier whose line requirement is softer than the trial table suggests: the phase 2 portion states patients may be systemic-treatment naive, and its ECOG ceiling is 2 rather than 1. It runs at a single site, MD Anderson in Houston, and prior gemcitabine is disqualifying in phase 2, so giving gemcitabine/docetaxel off-protocol first would close it. Abemaciclib is FDA-approved in breast cancer, so an off-label route exists, but with

no sarcoma data outside this study a payer will not fund it.

Next steps

- 1. Call MD Anderson at 866-632-6789 about NCT06498648 phase 2 and ask directly whether the treatment-naive stratum is currently open and whether an ECOG of 2 would still screen.
- 2. Do this before starting any gemcitabine-containing regimen, since prior gemcitabine disqualifies.
- 3. Document the ECOG at the same visit; the criterion is a measured score, not an assumed one.
- 4. Plan for travel to Houston and ask the site about screening turnaround relative to the LMS-04 start date.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06498648	2	recruiting	likely	Site Public Contact; askmdanderson@mdanderson.org; 866-632-6789

Manufacturer / sponsor contact

- **Company:** Eli Lilly and Company
- **Med info phone:** 1-800-545-5979
- **Product info:** <https://www.verzenio.com>
- **Notes:** Phone from the current Verzenio FDA label (1-800-LillyRx).

Payer / coverage: On trial the study drug is supplied; the chemotherapy backbone bills as usual care. Off-protocol abemaciclib in sarcoma has no compendium support and will be denied.

Notes: Reachable before LMS-04 starts. That distinguishes it from almost every other experimental option in this dossier.

10. Abemaciclib + gemcitabine and docetaxel

Aliases: Verzenio, LY2835219, abemaciclib-gemcitabine, abemaciclib, gemcitabine, docetaxel

Modality: small_molecule · **Regulatory:** Abemaciclib (Verzenio) FDA-approved in HR-positive HER2-negative breast cancer. No sarcoma indication. Gemcitabine and docetaxel are generic. · **Geographic scope:** United States; single site, MD Anderson, Houston TX · **Verified:** 2026-08-24

Supersedes the earlier access row for this intervention, adding the eligibility gate that row missed: entry requires intact Rb expression by immunohistochemistry on the baseline biopsy or archived tumour, assessed at MD Anderson with a CLIA-certified antibody. That is the exact opposite gate from the RB1-negative CDK7 protocol at the same institution, so one Rb stain on the tissue-order-B block routes her between the two trials: Rb intact points here, Rb lost points to REC-617 after a first line. The first-line opening stands, and so does the trap: the phase 2 portion admits systemic-treatment-naive patients, prior gemcitabine is disqualifying, and the ECOG ceiling is 2.

Next steps

- 1. Order Rb by IHC on the tissue-order-B block now. One stain routes her between this trial (Rb intact, first-line

eligible today) and REC-617 (Rb lost, after one prior line). The block cut was already priced; this adds no tissue cost, only the instruction to read the result against both protocols.

2. Call MD Anderson at 866-632-6789 and ask whether the site's own G3-245 assay must be run or a CLIA outside result is accepted, and whether the treatment-naïve stratum is open.
3. Do this before any gemcitabine-containing regimen; prior gemcitabine closes the phase 2 permanently. LMS-04 contains no gemcitabine and preserves eligibility.
4. Document the ECOG; ceiling 2.
5. Plan Houston travel and ask about screening turnaround against the LMS-04 start date.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06492548	1/2	recruiting	unconfirmed	Site Public Contact; askmdanderson@mdanderson.org; 866-632-6789

Manufacturer / sponsor contact

- **Company:** Eli Lilly and Company
- **Med info phone:** 1-800-545-5979
- **Product info:** <https://www.verzenio.com>
- **Notes:** Phone from the current Verzenio FDA label (1-800-LillyRx).

Payer / coverage: On trial the study drug is supplied and the chemotherapy backbone bills as usual care. Off-protocol abemaciclib in sarcoma has no compendium support and will be denied.

Notes: Supersedes row

ulms-highgrade-pulm-mets-tp53-smarcb1-hla-a2-w9t4-access-abemaciclib-gemcitabine-docetaxel (August 2026). What changed: the intact-Rb-by-IHC entry requirement was missing from the earlier row, and it converts Rb IHC from a nice-to-have stain into an access action that decides which of two MD Anderson protocols is even applicable. Reachable before LMS-04 starts, conditional on the Rb result.

11. Afamitresgene autoleucel (MAGE-A4 TCR-T, TECELRA)

Aliases: TECELRA, ADP-A2M4, afami-cel, afamitresgene-magea4-tcrt, ct-antigen-target-biology

Modality: cell_therapy · **Regulatory:** FDA-approved (TECELRA) for adults and patients 12 and older with unresectable or metastatic synovial sarcoma who have received prior chemotherapy, are HLA-A*02:01P/02:02P/02:03P/02:06P positive, and whose tumour expresses MAGE-A4 by an FDA-approved or cleared companion diagnostic. Label revised mid-2026 with the accelerated-approval statement removed. · **Guidelines:** Recognised in synovial sarcoma; no leiomyosarcoma recommendation · **Geographic scope:** US authorised treatment centres, synovial sarcoma only · **Verified:** 2026-08-24

Her HLA-A*02:01 clears the allele gate, but TECELRA's label is synovial sarcoma only, and uterine leiomyosarcoma is outside it. An autologous engineered T-cell product is not something a clinician writes off-label the way they would a tablet: authorised treatment centres order it against the label indication and its companion diagnostics. Record this as a closed on-label door for her rather than an option awaiting a MAGE-A4 stain.

Next steps

1. Do not present this as an open option on the strength of the HLA type alone. The allele is necessary and nowhere near sufficient.
2. If MAGE-A4 IHC is being ordered anyway as part of tissue order C, order it for the trial-based MAGE-A4 programs (CDR404, and the SURPASS product if it reopens), not for TECELRA.
3. Call USWM at 1-855-246-9232 only if a non-synovial cohort or a new protocol opens.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04024768		active not recruiting	no	—
NCT06647572		available	no	Clinical Scientist; francine.brophy@usworldmeds.com; 866-750-8796

Manufacturer / sponsor contact

- **Company:** USWM CT, LLC (US WorldMeds); developed by Adaptimmune Therapeutics
- **Med info phone:** 1-855-246-9232
- **Compassionate use email:** francine.brophy@usworldmeds.com
- **Notes:** Medical information phone taken verbatim from the current TECELRA FDA label. The email is the registry contact for the nonconforming-product expanded access protocol, not a general compassionate-use address.

Payer / coverage: A histology outside the label on a product of this cost will not be funded, and authorised treatment centres are unlikely to accept an order that does not match the companion-diagnostic pathway.

Notes: Explicitly recorded as not accessible on-label for her histology, per the case instruction, rather than left looking like a door held open by a pending stain.

12. All-trans retinoic acid + cemiplimab

Aliases: ATRA, tretinoin, Libtayo, cemiplimab-rwlc

Modality: combination · **Regulatory:** ATRA (tretinoin) FDA-approved in acute promyelocytic leukaemia. Cemiplimab (Libtayo) FDA-approved in cutaneous squamous cell carcinoma, basal cell carcinoma and NSCLC. Neither is approved in sarcoma. · **Geographic scope:** United States; single site, Columbus OH · **Verified:** 2026-08-24

Recruiting at Ohio State for leiomyosarcoma with no cap on prior lines, and its ECOG ceiling is 2. The gate is the other direction: patients must have received standard-of-care chemotherapy, so this opens after induction rather than before. Both components are approved drugs, so an off-label version is technically prescribable, but the combination rationale is trial-stage and no payer will fund cemiplimab in sarcoma.

Next steps

1. Email OSUCCCClinicaltrials@osumc.edu after the first chemotherapy line to confirm the cohort is still open.
2. Ask whether PRAME expression is being collected, since the retinoid rationale runs through PRAME and she may already have the stain.
3. Do not pursue the off-label version; the trial is the only sensible route.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06528769		recruiting	no	The Ohio State University Comprehensive Cancer Center; OSUCCCClinicaltrials@osumc.edu; 800-293-5066

Manufacturer / sponsor contact

- **Company:** Regeneron Pharmaceuticals, Inc. (cemiplimab)
- **Med info phone:** 1-877-542-8296
- **Notes:** Phone from the current Libtayo FDA label.

Payer / coverage: Off-label cemiplimab in leiomyosarcoma has no compendium support and would be denied. On trial the drugs are supplied.

13. Dostarlimab

Aliases: Jemperli, TSR-042

Modality: monoclonal_antibody · **Regulatory:** FDA-approved for dMMR recurrent or advanced solid tumours that have progressed on prior treatment, and in dMMR/MSI-H endometrial cancer. No sarcoma-specific indication.
· **Geographic scope:** US-wide, contingent on a dMMR result · **Verified:** 2026-08-24

Jemperli carries a tumour-agnostic accelerated approval for mismatch-repair-deficient recurrent or advanced solid tumours after prior therapy. MMR has never been tested on her tissue. It is rarely positive in uLMS but the IHC is cheap, so close the question rather than assume it.

Next steps

1. Add MMR IHC (MLH1, PMS2, MSH2, MSH6) to tissue order B; it rides the same block cut as INI1 and costs almost nothing extra.
2. If dMMR is found, this becomes a real post-induction option and GSK medical information at 1-888-825-5249 can supply the label package.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02715284		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** GlaxoSmithKline LLC (Tesaro)
- **Med info phone:** 1-888-825-5249
- **Product info:** <https://www.jemperli.com>
- **Notes:** Phone from the current Jemperli FDA label.

Payer / coverage: With dMMR documented, this is an on-label claim after prior therapy. Without it there is nothing to file.

14. Doxorubicin + ifosfamide (intensified)

Aliases: doxorubicin, ifosfamide, Ifex, EORTC 62012

Modality: small_molecule · **Regulatory:** Both FDA-approved, generic; no uterine leiomyosarcoma indication on either label. · **Guidelines:** NCCN soft tissue sarcoma first-line option where a higher response rate is the goal · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Both agents are generic and available at any sarcoma center; the combination is standard practice supported by EORTC 62012 even though neither label names uterine leiomyosarcoma. Nothing blocks access. The reasons to prefer or avoid it are toxicity and the mesna/hydration burden, not availability.

Next steps

1. Same baseline cardiac and pulmonary workup as any anthracycline plan.
2. If considered, weigh against the cumulative-doxorubicin ceiling that also gates the in vivo lung perfusion trial.

Payer / coverage: Generic; no meaningful coverage barrier.

15. Doxorubicin + pembrolizumab (first-line chemo-immunotherapy)

Aliases: doxorubicin, Keytruda, pembrolizumab

Modality: combination · **Regulatory:** Doxorubicin generic; pembrolizumab FDA-approved in other indications. The combination has no sarcoma indication. · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Both components are approved, so a first-line doxorubicin plus pembrolizumab regimen is prescribable off-label, but the supporting data is a phase 1/2 and there is no trial in this dossier offering it on protocol. Compared with the planned LMS-04 regimen it trades a phase 3 result for a phase 2 one, which is the reason to be cautious rather than any access barrier.

Next steps

1. Not a realistic first-line substitution for LMS-04 on access grounds.
2. If chemo-immunotherapy is genuinely wanted, the ivonescimab trial is the better-supported route to a PD-1 axis drug in this window.

Manufacturer / sponsor contact

- **Company:** Merck Sharp & Dohme LLC (pembrolizumab)
- **Med info phone:** 1-877-888-4231
- **Product info:** <https://www.keytruda.com>

Payer / coverage: Off-label pembrolizumab first line in sarcoma without an MSI or TMB result is very unlikely to clear.

16. Doxorubicin + trabectedin induction, then trabectedin maintenance (LMS-04)

Aliases: doxorubicin, Adriamycin, trabectedin, ET-743, Yondelis, LMS-04, doxorubicin-trabectedin-lms04, doxorubicin-backbone

Modality: small_molecule · **Regulatory:** Doxorubicin: FDA-approved, generic. Trabectedin (Yondelis): FDA-approved 2015 for unresectable or metastatic liposarcoma or leiomyosarcoma after a prior anthracycline-containing regimen. The first-line combination sits outside that label. · **Guidelines:** Doxorubicin-based first-line therapy is standard in NCCN soft tissue sarcoma; the specific doxorubicin + trabectedin pairing follows LMS-04. Category not independently verified this run. · **Geographic scope:** US-wide; any sarcoma-capable infusion center · **Verified:** 2026-08-24

Both drugs are stocked at any sarcoma infusion center, so delivery is not the problem. The problem is the label: Yondelis is approved for leiomyosarcoma only after a prior anthracycline-containing regimen, and she is anthracycline-naïve, so the planned first-line pairing is off-label prescribing carried by the LMS-04 phase 3 result. Build the prior authorization as an off-label first-line request from the outset rather than filing it and waiting for the denial.

Next steps

1. Document an ECOG score at the next clinic visit. It is undocumented, it gates most of the trial options in this dossier, and it costs nothing.
2. Get the baseline echocardiogram with LVEF, hepatic and renal panels, CBC read against altitude-adjusted haemoglobin references, and PFTs with DLCO and resting oximetry measured at her home altitude.
3. File the trabectedin prior authorization as an off-label first-line request citing LMS-04, before cycle 1 rather than after a denial.
4. Arrange central venous access; trabectedin runs as a 24-hour infusion.
5. For label or coverage questions call Janssen medical information at 1-800-526-7736.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02937358		completed	yes	—

Manufacturer / sponsor contact

- **Company:** Janssen Products, LP (Johnson & Johnson); trabectedin originator PharmaMar
- **Med info phone:** 1-800-526-7736
- **Product info:** <https://www.yondelis.com>
- **Notes:** Phone taken verbatim from the current FDA label: 'For more information, call 1-800-526-7736 or go to

Payer / coverage: Doxorubicin is generic and uncontested. Trabectedin is the expensive component and the label's prior-anthracycline requirement is the hook a payer will pull on. Cite the LMS-04 primary report and the final overall survival analysis plus whatever compendium listing the treating pharmacy can pull. Sarcoma centers win this appeal routinely, but only when it is filed as an off-label first-line request.

Notes: Starting this regimen is the event that closes the first-line window. Several trials in this dossier admit only treatment-naïve or one-prior-line patients, and a few close permanently once cumulative doxorubicin accumulates. Sequence the diagnostics and the trial outreach before cycle 1, not after.

17. Eribulin mesylate

Aliases: Halaven, E7389

Modality: small_molecule · **Regulatory:** FDA-approved for unresectable or metastatic liposarcoma after a prior anthracycline-containing regimen, and for metastatic breast cancer. Not approved for leiomyosarcoma. ·

Guidelines: Listed in NCCN soft tissue sarcoma; the leiomyosarcoma evidence is weaker than the liposarcoma evidence · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Halaven is approved in liposarcoma after an anthracycline, not in leiomyosarcoma, and the phase 3 that supported it found the benefit sat in the liposarcoma subgroup. So use here is off-label on a weak histology rationale. Available at any infusion center once a payer clears it, which is the whole obstacle.

Next steps

1. Defer. Revisit after anthracycline and gemcitabine-based lines.
2. If pursued, call Eisai medical information at 1-877-873-4724 for the label and subgroup data before filing.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT01327885		completed	no	—

Manufacturer / sponsor contact

- **Company:** Eisai Inc.
- **Med info phone:** 1-877-873-4724
- **Product info:** <https://www.halaven.com>
- **Notes:** Phone from the current Halaven FDA label.

Payer / coverage: Expect an off-label denial on first pass given the liposarcoma-restricted label and the negative leiomyosarcoma subgroup. Worth pursuing only after the better-supported options are exhausted.

18. Gemcitabine + dacarbazine

Aliases: gemcitabine, dacarbazine, DTIC, GEIS

Modality: small_molecule · **Regulatory:** Both FDA-approved, generic; no sarcoma indication on either label. · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Generic pair, orderable anywhere, off-label in sarcoma but supported by a randomised Spanish GEIS phase 2. Access is a non-issue; this sits in the later-line queue behind the anthracycline and gemcitabine/docetaxel options.

Next steps

1. No action now. Revisit at progression.

Payer / coverage: Generic; no barrier.

19. Gemcitabine + docetaxel

Aliases: gemcitabine, Gemzar, docetaxel, Taxotere

Modality: small_molecule · **Regulatory:** Both FDA-approved for other indications and available as generics; neither carries a sarcoma indication. · **Guidelines:** Long-established NCCN soft tissue sarcoma option for LMS · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Two generics, stocked everywhere, given in uLMS as long-standing off-label practice that payers do not fight. It is the first-line fallback if the baseline LVEF or her pulmonary reserve at 8,500 ft rules out full-dose doxorubicin. There is also a first-line-eligible randomised trial built on this backbone at MD Anderson (see the abemaciclib row), which is worth a call before committing to the regimen off-protocol.

Next steps

1. Hold as the backup plan pending the LVEF and PFT results that gate doxorubicin.
2. Before giving gemcitabine off-protocol, call MD Anderson at 866-632-6789 about NCT06498648, which requires gemcitabine-naive status in its phase 2 portion.
3. If used off-protocol, plan G-CSF support per the GOG dosing experience.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06498648	1/2	recruiting	likely	Site Public Contact; askmdanderson@mdanderson.org; 866-632-6789

Payer / coverage: Generic pricing keeps prior-authorization friction near zero; compendium support for sarcoma use is old and settled.

20. Gemcitabine + docetaxel +/- bevacizumab (GOG-0250)

Aliases: gemcitabine, docetaxel, bevacizumab, Avastin, GOG-0250

Modality: combination · **Regulatory:** Gemcitabine and docetaxel generic; bevacizumab FDA-approved in several solid tumours with multiple biosimilars. No uterine leiomyosarcoma indication. · **Geographic scope:** US-wide · **Verified:** 2026-08-24

All three drugs are approved and biosimilar bevacizumab is cheap, so nothing blocks the order. The reason to leave bevacizumab out is that GOG-0250 was negative: adding it to gemcitabine and docetaxel in uterine leiomyosarcoma did not improve outcomes. A payer that reads the trial will decline, and they would be right to.

Next steps

1. Use gemcitabine and docetaxel without bevacizumab if this backbone is chosen.
2. Recorded so the negative trial is visible in the access guide rather than absent from it.

Manufacturer / sponsor contact

- **Company:** Genentech, Inc. (Avastin) plus multiple biosimilar manufacturers
- **Notes:** No bevacizumab-specific medical information line was verified this run; route questions through the treating institution's pharmacy, which will be dispensing a biosimilar in any case.

Payer / coverage: The bevacizumab component has a negative randomised trial behind it in this exact histology. Do not build a request around it.

21. Gemcitabine + nab-sirolimus

Aliases: ABI-009, Fyarro, nab-sirolimus, sirolimus protein-bound

Modality: small_molecule · **Regulatory:** Nab-sirolimus (FYARRO) FDA-approved for locally advanced unresectable or metastatic malignant PEComa. Gemcitabine generic. The combination is investigational. · **Geographic scope:** United States; trial single-site Houston TX, expanded access nationally through the sponsor · **Verified:** 2026-08-24

The MD Anderson phase 1 requires either leiomyosarcoma with one prior systemic therapy or a TSC1/TSC2 loss-of-function alteration, and its ECOG ceiling is 2. Nab-sirolimus is separately FDA-approved in PEComa and Aadi maintains a registered expanded-access protocol covering malignancies with relevant genetic mutations or mTOR pathway activation, which is a genuine alternative route if a TSC alteration turns up on tissue sequencing. TSC status is currently unmeasured.

Next steps

1. Include TSC1 and TSC2 in tissue order A. It is a standard part of comprehensive panels and costs nothing extra to report.
2. If a TSC alteration is found, call Aadi at 888-246-2234 and ask specifically about eligibility for NCT03817515.
3. The MD Anderson combination trial requires one prior line for leiomyosarcoma patients, so it is a post-induction option regardless.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06308419		recruiting	no	Elise Nassif, MD; efnassif@mdanderson.org; (281) 460-0607
NCT03847515		approved for marketing	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Aadi Bioscience, Inc.
- **Med info phone:** 888-246-2234
- **Product info:** <https://www.aadibio.com>
- **Notes:** Phone from the current FYARRO FDA label (1-888-BIO-AADI).

Payer / coverage: Off-label nab-sirolimus outside PEComa needs a documented TSC1/TSC2 alteration or mTOR pathway activation to have any chance. The expanded-access protocol is the cleaner route if she qualifies, because it does not depend on a payer.

22. Larotrectinib (TRK inhibition, NTRK-fusion gated)

Aliases: Vitrakvi, LOXO-101, TRK inhibitor

Modality: small_molecule · **Regulatory:** FDA-approved (Vitrakvi) for adult and paediatric solid tumours with an NTRK gene fusion without a known acquired resistance mutation, that are metastatic or where surgical resection is likely to cause severe morbidity, and that have no satisfactory alternative treatment or have progressed following treatment. · **Guidelines:** Tumour-agnostic NCCN recognition for NTRK fusion-positive disease · **Geographic scope:** US-wide, contingent on a documented NTRK fusion · **Verified:** 2026-08-24

Vitrakvi is tumour-agnostic on-label for NTRK gene fusion-positive solid tumours, so if a fusion exists she is on-label, not off-label, and coverage follows. Bayer also maintains a registered expanded-access protocol for NTRK fusion cancers. Fusion status is unmeasured: plasma is insensitive to fusions at low tumour fraction, so a negative liquid biopsy settles nothing and an RNA-based tissue panel is the test that does.

Next steps

1. Include RNA-based fusion calling in tissue order A. Pan-TRK IHC is a screen only and a positive would still need fusion-capable confirmation, which costs an extra two weeks.
2. Use the 2026 lung core rather than the 2024 block as the RNA input; RNA quality degrades in older blocks.
3. If a fusion is found, call Bayer at 1-888-842-2937 about both the standard prescription route and NCT03025360.
4. The prior probability of an NTRK fusion in uterine leiomyosarcoma is very low. This is worth closing cheaply, not worth waiting on.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02526431		completed	no	—

NCT03025360	approved for marketing	unconfirmed	—
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Manufacturer / sponsor contact

- **Company:** Bayer HealthCare Pharmaceuticals Inc.; developed with Loxo Oncology
- **Med info phone:** 1-888-842-2937
- **Product info:** <https://www.vitrakvi.com>
- **Notes:** Phone from the current Vitrakvi FDA label.

Payer / coverage: With a documented fusion this is an on-label claim. The label's 'no satisfactory alternative treatment' language means a payer may still ask why standard chemotherapy is not being used first, so the request is cleaner after a line of therapy than before one.

23. Lurbinectedin + doxorubicin

Aliases: Zepzelca, PM01183, SaLuDo

Modality: small_molecule · **Regulatory:** Lurbinectedin (Zepzelca) FDA-approved for metastatic small cell lung cancer. No sarcoma indication. · **Geographic scope:** US and EU sites, all closed to new enrolment · **Verified:** 2026-08-24

The trial that would have fit her almost perfectly, SaLuDo, is a first-line anthracycline-naïve leiomyosarcoma study and it has stopped enrolling. Lurbinectedin itself is FDA-approved in small cell lung cancer, so an off-label request is technically possible, but with the phase 3 read-out not yet public there is no compendium support to hang a payer appeal on. Practically this is not a route for her right now.

Next steps

1. Confirm with the SaLuDo sites that accrual is truly closed rather than paused before writing it off; the registry status is 'active, not recruiting'.
2. Call Jazz medical information at 1-800-520-5568 to ask whether any expanded-access or continued-access provision exists for the sarcoma program.
3. Otherwise treat as unavailable in the first-line window.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06038290		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Jazz Pharmaceuticals, Inc. (US); originator PharmaMar
- **Med info phone:** 1-800-520-5568
- **Product info:** <https://www.zepzelca.com>
- **Notes:** Phone from the current Zepzelca FDA label.

Payer / coverage: Off-label in sarcoma with no compendium listing and no published phase 3; expect denial.

Revisit if SaLuDo reports positive and a listing follows.

24. Nivolumab

Aliases: Opdivo, BMS-936558

Modality: monoclonal_antibody · **Regulatory:** FDA-approved across multiple solid tumours; no sarcoma indication. · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Approved in many tumour types, so prescribable off-label, but the dedicated uterine leiomyosarcoma phase 2 was negative and there is no biomarker in this case pointing the other way. Access is easy; the reason not to pursue it is the evidence, not the route.

Next steps

1. No action. Recorded so the negative is visible rather than absent.

Payer / coverage: Off-label single-agent PD-1 blockade in uLMS against a negative phase 2 will be denied and should be.

25. Nivolumab +/- ipilimumab

Aliases: Opdivo, Yervoy, A091401

Modality: monoclonal_antibody · **Regulatory:** Both FDA-approved across multiple solid tumours; no sarcoma indication. · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Both drugs are approved and the combination is prescribable off-label. Alliance A091401 showed the combination beat single-agent nivolumab in sarcoma overall, but the leiomyosarcoma contribution was thin and the toxicity is not. This is a later-line consideration with a real off-label coverage fight attached.

Next steps

1. Defer to later lines.
2. If pursued, file with the A091401 data and an explicit toxicity-monitoring plan.

Payer / coverage: Dual checkpoint blockade off-label is expensive and will draw a full review. A091401 is the citation, but it is a randomised phase 2 without a sarcoma indication behind it.

26. Olaparib + temozolomide

Aliases: Lynparza, AZD2281, Temodar, TMZ, NCI 10250

Modality: small_molecule · **Regulatory:** Olaparib (Lynparza) FDA-approved in BRCA-mutated ovarian, breast, pancreatic and prostate cancers. Temozolomide approved in glioma. Neither is approved in sarcoma. ·

Geographic scope: Off-label US-wide; both trials closed to enrolment · **Verified:** 2026-08-24

Both drugs are approved and prescribable, and the uterine leiomyosarcoma phase 2 that generated the signal reported a real response rate, but both trials of this combination are now closed to accrual. So the only live route is an off-label prescription, and that request will go much better with a tissue HRD or HRR result in hand than with the current plasma panel, which does not settle homologous recombination status.

Next steps

- 1. Send tissue order A (DNA plus RNA comprehensive profiling with an HRD or genomic-instability score) before anything else; it is the gate on this whole branch.
- 2. Order the germline homologous-recombination panel alongside rather than reflexively after a somatic finding.
- 3. Re-check NCT05432791 site status at progression; a 145-site cooperative-group study can reopen.
- 4. If pursuing off-label, call AstraZeneca medical information at 1-800-236-9933 and build the file around the tissue HRD result plus the NCI 10250 uLMS data.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03820019		active not recruiting	no	—
NCT05422791		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** AstraZeneca Pharmaceuticals LP
- **Med info phone:** 1-800-236-9933
- **Product info:** <https://www.lynparza.com>
- **Notes:** Phone from the current Lynparza FDA label.

Payer / coverage: Olaparib off-label in sarcoma is a hard sell without a documented HRR alteration or HRD score. A somatic BRCA1/2 or high genomic-instability result on tissue changes the conversation entirely, which is one more reason tissue order A should go out now rather than after induction.

Notes: Post-induction option. Nothing here is reachable in the first-line window except the tissue testing that gates it.

27. PARP inhibition in BRCA2-altered uterine leiomyosarcoma

Aliases: olaparib, Lynparza, niraparib, rucaparib, HRD

Modality: small_molecule · **Regulatory:** PARP inhibitors approved across BRCA-mutated ovarian, breast, pancreatic and prostate cancers. No sarcoma indication. · **Geographic scope:** US-wide, contingent on a tissue HRR result · **Verified:** 2026-08-24

Same access mechanics as the olaparib combination row: the drug is prescribable, the histology is not on any label, and the whole question turns on whether a tissue HRR alteration exists. The plasma panel reported no BRCA1/2 alteration, but plasma negatives at low tumor fraction do not close this.

Next steps

1. Order tissue HRR sequencing with a genomic-instability score as part of tissue order A.
2. Add the germline homologous-recombination panel in parallel.
3. Only pursue a payer request once a tissue result exists.

Manufacturer / sponsor contact

- **Company:** AstraZeneca Pharmaceuticals LP (olaparib)
- **Med info phone:** 1-800-236-9933
- **Product info:** <https://www.lynparza.com>

Payer / coverage: A documented somatic or germline BRCA2 alteration is the difference between a fundable request and a reflex denial.

28. Pegylated liposomal doxorubicin (maintenance)

Aliases: PLD, Doxil, Caelyx, MELODY

Modality: small_molecule · **Regulatory:** FDA-approved (Doxil) for ovarian cancer after platinum, AIDS-related Kaposi sarcoma, and multiple myeloma. No soft tissue sarcoma indication. · **Geographic scope:** Off-label US-wide; the MELODY trial is Taiwan-only · **Verified:** 2026-08-24

Doxil is FDA-approved in ovarian cancer, myeloma and Kaposi sarcoma, so off-label maintenance use in soft tissue sarcoma is prescribable in the US, though without guideline backing. The trial testing exactly this question (MELODY) is Taiwan-only. Either way this is a post-induction question, not a first-line one.

Next steps

1. Park until the maintenance phase of LMS-04 is underway and the question is live.
2. If pursued then, note the cumulative doxorubicin ceiling already consumed by induction.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06921637		recruiting	no	Hui-Jen Tsai, MD; hjtsai@nhri.edu.tw ; +886-6-7000123

Manufacturer / sponsor contact

- **Company:** Baxter Healthcare Corporation (Doxil); generic PLD also marketed
- **Med info phone:** 1-866-888-2472
- **Notes:** Phone from the current Doxil FDA label.

Payer / coverage: Off-label maintenance in sarcoma has no compendium listing; a payer will ask what the anthracycline maintenance evidence is, and the honest answer today is a Taiwanese phase 2 still enrolling.

29. Pembrolizumab

Aliases: Keytruda, MK-3475, checkpoint-blockade-ulms

Modality: monoclonal_antibody · **Regulatory:** FDA-approved across many tumour types, including tumour-agnostic indications for MSI-high/dMMR and for TMB-high (≥ 10 mut/Mb) solid tumours after prior therapy. No sarcoma-specific indication. · **Guidelines:** Tumour-agnostic NCCN recognition for MSI-H/dMMR and TMB-high; no LMS-specific recommendation · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Keytruda carries tumour-agnostic approvals for microsatellite-instability-high and TMB-high solid tumours, so if tissue testing returns either, she is on-label rather than off-label and coverage follows. Neither has been measured; the plasma panel did not report TMB or MSI. Without one of those results, use in leiomyosarcoma is off-label against a negative SARC028 LMS cohort, which payers know.

Next steps

1. Include validated tissue TMB and MSI reporting in tissue order A, and MMR IHC in tissue order B. Both are cheap and both can open an on-label door.
2. If TMB ≥ 10 or MSI-high, revisit after the first chemotherapy line, since the agnostic indication requires prior therapy.
3. If both are negative, treat single-agent checkpoint blockade in uLMS as closed and put the effort into the antigen-directed branches instead.
4. Merck medical information: 1-877-888-4231.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02628067		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Merck Sharp & Dohme LLC
- **Med info phone:** 1-877-888-4231
- **Product info:** <https://www.keytruda.com>
- **Notes:** Phone from the current Keytruda FDA label.

Payer / coverage: MSI-high or TMB ≥ 10 mut/Mb on a validated tissue assay converts this from an off-label appeal into an on-label claim. The tumour-agnostic TMB indication also requires prior therapy, so it is a post-induction claim even if TMB comes back high.

Notes: The tumour-agnostic route is post-induction by label design. The testing that determines whether the route exists at all should still happen now.

30. Tazemetostat

Aliases: Tazverik, EPZ-6438, E7438, tazemetostat-ezh2, smarcb1-ini1-target-biology

Modality: small_molecule · **Regulatory:** FDA-approved (accelerated) for metastatic or locally advanced epithelioid sarcoma not eligible for complete resection in patients 16 and older, and for relapsed/refractory follicular lymphoma. Not approved in leiomyosarcoma. · **Guidelines:** NCCN recognition is in epithelioid sarcoma; off-guideline for uterine leiomyosarcoma · **Geographic scope:** US-wide as an off-label prescription; no open expanded-access program · **Verified:** 2026-08-24

Tazverik is approved in epithelioid sarcoma, not uterine leiomyosarcoma, so every route here is off-label or trial-based cross-histology extrapolation. The gate is INI1 IHC on tumour tissue: a plasma SMARCB1 frameshift is a hypothesis, and no payer and no trial will act on it. Both tazemetostat expanded-access programs are listed as no longer available, so if INI1 loss is confirmed the realistic path is an off-label prescription with an appeal built on the INI1-negative basket data.

Next steps

1. Order INI1/SMARCB1 IHC (BAF47 clone) on tumour tissue as part of tissue order B. That single stain decides whether this branch exists.
2. Order it on both the 2024 uterine block and the 2026 pulmonary core; discordance between primary and metastasis is a known problem and one negative on a small core is not reassuring.
3. If INI1 is lost, call Epizyme/Ipsen medical information at 855-463-5127 and ask what managed-access or named-patient route currently exists, since both registered expanded-access programs are closed.
4. In parallel, ask about NCT07303387 (valemistat), which requires a bi-allelic homozygous deletion her plasma frameshift does not establish.
5. If INI1 is intact, close this branch and say so in the record rather than leaving it open.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02621950		completed	no	—
NCT03874455		no longer available	no	—
NCT04225429		no longer available	no	—

Manufacturer / sponsor contact

- **Company:** Epizyme, Inc., an Ipsen Company
- **Med info phone:**855-463-5127
- **Notes:** Phone taken verbatim from the current Tazverik FDA label: 'For more information, contact Epizyme at 855-463-5127.' No separate compassionate-use email was verifiable this run; ask the medical information line for the current managed-access route.

Payer / coverage: Off-label EZH2 inhibition in uLMS is expensive and will be reviewed carefully. A documented loss of nuclear INI1 staining, ideally on both the 2024 uterine block and the 2026 lung core, is what makes the appeal arguable. Without it there is no filing worth making.

Notes: The load-bearing honesty in this case: SMARCB1 loss is uncommon in uterine leiomyosarcoma, and the call came from plasma at unstated variant fraction. The prior is low, the confirmatory test is cheap, and the branch should not be priced as if it were already true.

31. Tebentafusp (gp100 x CD3 ImmTAC)

Aliases: KIMMTRAK, IMCgp100, tebentafusp-tebn

Modality: bispecific_fusion · **Regulatory:** FDA-approved (KIMMTRAK) for HLA-A*02:01-positive adults with unresectable or metastatic uveal melanoma. No sarcoma indication. · **Geographic scope:** US-wide on label for uveal melanoma; the sarcoma trial is clear cell sarcoma only · **Verified:** 2026-08-24

KIMMTRAK is FDA-approved in HLA-A*02:01-positive unresectable or metastatic uveal melanoma, so the allele gate is met but the indication is not. The sarcoma study now recruiting is restricted to HMB-45-positive clear cell sarcoma, a different histology from hers. Note its oxygen criterion is written as an exclusion at saturation below 92% on room air, which is stricter than the 90% floor used elsewhere in this class.

Next steps

1. No realistic action. Included because the ImmTAC class precedent is cited in the dossier and a reader will ask whether the approved member is reachable.
2. The transferable point is the class oximetry threshold, which should be settled before any T-cell-engager screening.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06922442		recruiting	no	SARC Office; SARC045@sarcctrials.org; (734) 930-7600

Manufacturer / sponsor contact

- **Company:** Immunocore Commercial LLC
- **Med info phone:** 1-844-466-8661
- **Product info:** <https://www.kimmtrak.com>
- **Notes:** Phone from the current KIMMTRAK FDA label (1-844-IMMUNO1).

Payer / coverage: Off-label use outside uveal melanoma is very unlikely to be funded; gp100 is a melanocytic-lineage antigen with no rationale in leiomyosarcoma.

32. Temozolomide

Aliases: Temodar, TMZ

Modality: small_molecule · **Regulatory:** FDA-approved for glioma indications; generic. No sarcoma indication. · **Geographic scope:** US-wide · **Verified:** 2026-08-24

Oral generic, easy to get, off-label in sarcoma. Its real relevance in this case is as the partner in the olaparib combination, where the PARP inhibitor rather than temozolomide is the cost and coverage problem.

Next steps

1. Treat as an add-on to the olaparib decision rather than a separate access question.

Manufacturer / sponsor contact

- **Company:** Merck Sharp & Dohme LLC (Temodar brand); multiple generic manufacturers
- **Med info phone:** 1-877-888-4231
- **Product info:** <https://www.temodar.com>
- **Notes:** Phone from the current Temodar FDA label.

Payer / coverage: Generic oral; low friction. Pair the request with the olaparib filing so both are reviewed together.

33. Tiragolumab + atezolizumab

Aliases: Tecentriq, MTIG7192A, RO7092284, anti-TIGIT

Modality: monoclonal_antibody · **Regulatory:** Atezolizumab (Tecentriq) FDA-approved in several solid tumours including alveolar soft part sarcoma. Tiragolumab investigational, no approval. · **Geographic scope:** US, Canada, Australia; closed to new enrolment · **Verified:** 2026-08-24

The NCI study of this combination in SMARCB1- or SMARCA4-deficient tumours has stopped enrolling, and tiragolumab is investigational with no approval anywhere, so the combination is not obtainable. Atezolizumab alone is approved and could be prescribed off-label, but single-agent PD-L1 blockade in uLMS has the same negative evidence base as the other checkpoint rows.

Next steps

1. Ask the NCI trial referral line whether NCT05286801 has any plan to reopen; a 40-site study that closed to accrual sometimes adds cohorts.
2. Otherwise treat the TIGIT combination as unavailable and keep the INI1 result for whatever SWI/SNF study opens next.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05286801	1/2	active not recruiting	no	—

Payer / coverage: Off-label atezolizumab monotherapy in uLMS has no compendium support.

34. Tislelizumab + liposomal doxorubicin and ifosfamide

Aliases: Tevimbra, BGB-A317, tislelizumab, TAIS

Modality: combination · **Regulatory:** Tislelizumab (Tevimbra) FDA-approved in oesophageal squamous cell and gastric/gastroesophageal junction adenocarcinoma. No sarcoma indication. · **Geographic scope:** Mainland China only for the trial; tislelizumab is prescribable off-label in the US · **Verified:** 2026-08-24

Another first-line study that is single-site in China. Tislelizumab is FDA-approved in oesophageal and gastric indications, so an off-label US version of the regimen is technically prescribable, but there is no sarcoma evidence beyond this trial and no payer will fund it.

Next steps

1. No US access to the trial. Confirm before writing off, but the registry lists one Chinese site.
2. Do not pursue the off-label version; the chemo-immunotherapy question is better answered by the ivonescimab study, which is open to her.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06829986		recruiting	no	Xin Liu; jeanettexin@hotmail.com; +86 13761503356

Clinical trial (23)

35. ADCE-D01 (uPARAP/Endo180-directed antibody-drug conjugate)

Aliases: ADCE-D01, uPARAP, Endo180, MRC2

Modality: antibody_drug_conjugate · **Regulatory:** Investigational; no approval anywhere. · **Geographic scope:** United States and Western Europe · **Verified:** 2026-08-24

A sarcoma-specific first-in-human ADC with US and European sites, recruiting. It requires one to two prior lines of cytotoxic therapy for metastatic disease and ECOG 0-1, so it opens after induction and closes if she accumulates more than two lines. That narrow window is worth marking on the calendar now.

Next steps

1. Email rene.smrcka@adcendo.com now to ask which US sites are screening and what the current slot situation is; the answer will still be useful in six months.
2. The eligible window is one to two prior lines. After LMS-04 she is at one, which is the best moment to approach.
3. Document the ECOG; the ceiling is 1.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06797299		recruiting	no	René Smrčka; rene.smrcka@adcendo.com; +45 3144 0653

Manufacturer / sponsor contact

- **Company:** Adcendo ApS
 - **Med info email:** rene.smrcka@adcendo.com
 - **Notes:** Contact captured verbatim from the NCT06797999 registry record.
-

36. ATR inhibition (ATRX / ALT-directed hypothesis)

Aliases: ATR inhibitor, ceralasertib, ALT, ATRX

Modality: small_molecule · **Regulatory:** No ATR inhibitor is approved in any indication. · **Geographic scope:** No identified open trial for this patient · **Verified:** 2026-08-24

No ATR-inhibitor trial surfaced in this case's trial screen, so there is no specific protocol to call, and no ATR inhibitor is approved anywhere. The underlying hypothesis is also shakier than it first looks: the original claim that ALT-positive cells are selectively ATR-inhibitor sensitive failed to replicate, and the sarcoma activity that has been seen came with gemcitabine and did not track ALT status.

Next steps

1. Include ATRX in tissue order A and, if available, ATRX loss by IHC in tissue order B. That is the cheap part and it informs biology regardless.
2. Re-screen for open ATR-inhibitor studies at progression rather than now; the field moves and this dossier's screen found nothing.
3. Do not weight this branch as though the ALT synthetic-lethality claim were established.

Notes: Flagged for the run log: the preclinical file carries this as an intervention id but the trial screen returned no matching protocol. If ATR inhibition is to be a real option it needs a fresh trial search.

37. Anlotinib (catequentinib, AL3818)

Aliases: AL3818, catequentinib, APROMISS

Modality: small_molecule · **Regulatory:** Approved in China (NMPA) for several indications including soft tissue sarcoma. No FDA or EMA approval. · **Geographic scope:** US, EU and China sites open for other indications; the leiomyosarcoma arm is suspended · **Verified:** 2026-08-24

The APROMISS phase 3 is still recruiting, but its leiomyosarcoma arm is marked closed and new recruitment to that indication suspended on the registry; only the broader 'any sarcoma or other solid tumour' indication remains open. The LMS arm also required at least one prior line of approved therapy and ECOG 0-1. Anlotinib is approved in China but not by FDA or EMA, so there is no off-label route in the US.

Next steps

1. Email Shiyings@advenchen.com to ask whether the leiomyosarcoma indication has reopened and whether Indication E would accept a leiomyosarcoma patient.
2. Either way this needs at least one prior line, so it is a post-induction question.
3. No US off-label route exists; do not present the Chinese approval as an access path.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03036819		recruiting	no	Shiying Clinical Trial Manager; Shiyings@advenchen.com; 8055301550

Manufacturer / sponsor contact

- **Company:** Advenchen Laboratories, LLC (US development); originator Chia Tai Tianqing
- **Med info email:** Shiyings@advenchen.com
- **Notes:** Contact captured verbatim from the NCT03016819 registry record; it is the trial manager rather than a general medical information line.

38. B7-H3-directed antibody-drug conjugates (MGC026; GSK5764227/HS-20093)

Aliases: MGC026, GSK5764227, HS-20093, risvutatug rezetecan, CD276, B7-H3

Modality: antibody_drug_conjugate · **Regulatory:** All investigational; no B7-H3 ADC is approved anywhere. · **Geographic scope:** United States, UK, Australia, Canada; the sarcoma-specific HS-20093 studies are China-based · **Verified:** 2026-08-24

Two of these are recruiting with US sites and both name sarcoma in their eligible tumour types, which makes the class one of the more realistic later-line routes here. Neither requires a B7-H3 result to enrol, but B7-H3 IHC is a cheap add to tissue order B and would tell her whether the target is even present. Both are dose-escalation studies with prior-line expectations, so this is post-induction.

Next steps

1. Add B7-H3 (CD276) IHC to tissue order B. It rides the same block cut as INI1 and MMR and tells her whether the class has a target here at all.
2. Read the larger 2024 resection rather than the lung core alone for B7-H3; expression is spatially heterogeneous.
3. After the first line, email info@macrogenics.com and GSKClinicalSupportHD@gsk.com to ask which US sites have open sarcoma slots.
4. Expect the sponsor to re-test on its own assay; a local IHC result is a reason to approach, not a substitute.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06242470		recruiting	unconfirmed	Global Trial Manager; info@macrogenics.com ; 301-251-5172
NCT06551142		recruiting	unconfirmed	US GSK Clinical Trials Call Center; GSKClinicalSupportHD@gsk.com ; 877-379-3718

NCT06691576	not yet recruiting	no	Xiaodong Tang, PhD; tang15877@126.com; 010-88324561
NCT06935409	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** MacroGenics, Inc. (MGC026); GlaxoSmithKline with Hansoh (GSK5764227/HS-20093)
- **Med info phone:** 301-251-5172
- **Med info email:** info@macrogenics.com
- **Notes:** MacroGenics contact captured verbatim from the NCT06242470 registry record. GSK US clinical trials call center: 877-379-3718, GSKClinicalSupportHD@gsk.com, from the NCT06551142 record.

39. Brenetafusp (IMC-F106C, PRAME x CD3 ImmTAC)

Aliases: IMC-F106C, PRAME ImmTAC, brenetafusp

Modality: bispecific_fusion · **Regulatory:** Investigational; no approval. Sponsor markets tebentafusp in the same ImmTAC class. · **Geographic scope:** International; closed to new enrolment · **Verified:** 2026-08-24

The PRAME ImmTAC phase 1/2 has stopped enrolling. It is an off-the-shelf infusion rather than a manufactured cell product, so it would have avoided the leukapheresis and manufacturing delay that makes TCR-T hard to fit into a first-line window. Worth one call to Immunocore about cohort reopening, since the sponsor has an active PRAME program.

Next steps

1. Call Immunocore at 1-844-466-8661 and ask whether any brenetafusp cohort is reopening or whether a successor PRAME study is registered.
2. If so, the PRAME stain from tissue order C is the prerequisite and should already be in hand.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04261266		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Immunocore Ltd
- **Med info phone:** 1-844-466-8661
- **Product info:** <https://www.kimmtrak.com>
- **Notes:** Phone from the current KIMMTRAK FDA label (1-844-IMMUNO1). Immunocore's general US medical information line; no brenetafusp-specific line is published.

40. CC-3 (B7-H3 x CD3 bispecific antibody)

Aliases: CC-3, CD276xCD3

Modality: bispecific_antibody · **Regulatory:** Investigational; no approval anywhere. · **Geographic scope:** Germany only; single site · **Verified:** 2026-08-24

The only clinical study of CC-3 runs at a single German academic site and its registered population is colorectal cancer, with the sarcoma work still preclinical. Nothing about this is reachable from the US today. It is in the dossier because the preclinical sarcoma data is genuinely interesting, not because there is a seat.

Next steps

1. No action. Recorded so the B7-H3 x CD3 branch shows as checked and closed rather than absent.
2. If a sarcoma cohort opens, kketi@med.uni-tuebingen.de is the registry contact.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05999396		recruiting	no	Juliane Walz, Prof. Dr.; kkti@med.uni-tuebingen.de; +49 7071 29

41. CDR404 (MAGE-A4 x CD3 T-cell engager)

Aliases: CDR404, MAGE-A4xCD3

Modality: bispecific_antibody · **Regulatory:** Investigational; no approval anywhere. · **Geographic scope:** United States and Western Europe; US sites open · **Verified:** 2026-08-24

Recruiting, open at US sites, and it takes MAGE-A4-expressing solid tumours in HLA-A*02:01 patients rather than a fixed histology list. Because it is an antibody-format engager rather than a cell product, there is no leukapheresis or manufacturing lead time. The blockers are MAGE-A4 expression, which is unmeasured, and an ECOG 0-1 ceiling against an undocumented score.

Next steps

1. Email CDR404-001_Study@CDR-Life.com and ask whether a systemic-therapy-naive patient can enrol, and which US sites are currently screening.
2. Ask what MAGE-A4 assay and cutoff the study uses and whether a local IHC result is accepted for pre-screening.
3. Order MAGE-A4 as part of tissue order C now; a stain that takes one to two weeks is the difference between screening in this window and screening after induction.
4. Document the ECOG.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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Manufacturer / sponsor contact

- **Company:** CDR-Life AG
- **Med info phone:** +41 44 515 7025
- **Med info email:** CDR404-001_Study@CDR-Life.com
- **Notes:** Contact captured verbatim from the NCT06402201 registry record.

Notes: Along with the ivonescimab study, this is one of the few investigational routes whose registry text does not obviously require a prior line. Confirm that with the sponsor rather than assuming it.

42. FAP-directed radioligand therapy ([225Ac]RTX-2358, [212Pb]Pb-PSV359, [177Lu]Lu-FAP-2286)

Aliases: RTX-2358, PSV359, FAP-2286, FAPI, fibroblast activation protein

Modality: radioligand · **Regulatory:** All investigational; no FAP-directed radioligand is approved anywhere. · **Geographic scope:** United States, Canada, Australia, Western Europe · **Verified:** 2026-08-24

Three recruiting programs, all with US sites, and all of them gate on a positive FAP imaging scan rather than on tissue: an FAPI PET for the Ratio study, a [203Pb]Pb-PSV359 SPECT/CT for Perspective, a [68Ga]Ga-FAP-2286 PET for Novartis. That imaging step is the access action, and it is not a routine scan outside these protocols. The Ratio study asks for relapsed and refractory sarcoma; all three cap ECOG at 0-1.

Next steps

1. Not accessible before induction. The Ratio study requires relapsed and refractory disease and the others expect prior lines.
2. When the time comes, the first call is about imaging: ask each site how a FAP PET is arranged and whether it happens before or after formal screening.
3. clinicaltrials@perspectivetherapeutics.com is the single most useful address here because the sponsor runs the imaging and therapy pair together across 11 US sites.
4. Document the ECOG; all three cap at 1.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07136565		recruiting	no	Jacqueline Banuelos; jbanuelosmurillo@mednet.ucla.edu; 310-869-7014

NCT06710256	recruiting	unconfirmed	ClinicalTrials at Perspectivetherapeutics; (206) 676-0900
NCT04930210	recruiting	unconfirmed	Novartis Pharmaceuticals; novartis.email@novartis.com; 1-888-689-6682

Manufacturer / sponsor contact

- **Company:** Ratio Therapeutics, Inc.; Perspective Therapeutics; Novartis Pharmaceuticals
- **Med info phone:** 1-888-689-6682
- **Med info email:** clinicaltrials@perspectivetherapeutics.com
- **Notes:** Novartis phone and Perspective email both captured verbatim from their respective registry records. Ratio Therapeutics posts no central contact; use the site contacts above.

43. Fluzoparib + camrelizumab with concurrent SBRT

Aliases: fluzoparib, SHR-3162, camrelizumab, SHR-1210, PRIMA

Modality: combination · **Regulatory:** Both approved in China; neither has FDA or EMA approval. · **Geographic scope:** Mainland China only · **Verified:** 2026-08-24

Single site in Shanghai. Neither fluzoparib nor camrelizumab is FDA-approved, so there is no off-label route either. The idea of pairing PARP inhibition with radiotherapy and checkpoint blockade is interesting given her oligometastatic-adjacent lung disease, but this particular protocol is out of reach.

Next steps

1. No US access. The transferable idea, SBRT to lung lesions combined with systemic therapy, can be pursued locally without this protocol.
2. See the SBRT row for the domestically available version of the radiotherapy component.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06024692		recruiting	no	Weibin Zhang, PhD, MD; zhangweibin10368@163.com; +8613501824630

44. IMA203 / IMA203CD8 (PRAME-directed TCR-T)

Aliases: IMA203, IMA203CD8, ACTengine, prame-directed-tcr-immTac

Modality: cell_therapy · **Regulatory:** Investigational; no approval anywhere. · **Geographic scope:** United

States and Germany; multiple US sites · **Verified:** 2026-08-24

This is recruiting, it is open in the US, and it accepts solid tumours generally rather than a fixed histology list, which makes it the most reachable engineered-cell option in the dossier. Two things stand between her and it: PRAME expression must be confirmed on Immatics' own RT-qPCR assay, and the protocol requires patients to have received or be ineligible for all available indicated standard-of-care treatment, which reads as post-induction rather than first-line. ECOG ceiling is 0-1, so the undocumented performance status matters here too.

Next steps

1. Email ctgovinquiries@immatics.com now, before induction, with two questions: does uterine leiomyosarcoma fall inside the current solid-tumour cohorts, and does 'received or not eligible for all available indicated standard of care' allow a patient who has declined first-line therapy.
2. Ask in the same message how PRAME screening is initiated and whether archival tissue from a 2024 resection is acceptable input for IMADetect.
3. Document the ECOG; this protocol caps at 0-1 and an assumed score will not screen.
4. Order PRAME as part of tissue order C so a local result is in hand when the sponsor assay is discussed.
5. Ask about resting room-air oxygen saturation requirements and flag the 8,500 ft residence in that first message.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03686124		recruiting	unconfirmed	Immatics US, Inc.; ctgovinquiries@immatics.com ; +1 346 204-5400

Manufacturer / sponsor contact

- **Company:** Immatics US, Inc.
- **Med info phone:** +1 346 204-5400
- **Med info email:** ctgovinquiries@immatics.com
- **Notes:** Contact captured verbatim from the NCT03686124 registry record.

Notes: Most likely a post-induction option given the standard-of-care-exhaustion criterion, but the screening conversation and the PRAME stain should both start in the first-line window because they take weeks.

45. In vivo lung perfusion with doxorubicin, plus metastasectomy

Aliases: IVLP, in vivo lung perfusion, isolated lung perfusion

Modality: surgery · **Regulatory:** Doxorubicin is approved; the regional perfusion delivery is investigational. · **Geographic scope:** Canada only; single site, Toronto · **Verified:** 2026-08-24

This is the closest anatomic fit in the dossier and it carries a sequencing conflict that a yes/no eligibility column cannot express. It is recruiting at a single Toronto centre with an ECOG ceiling of 2, but it excludes patients who have previously received more than 450 mg of doxorubicin, and a full anthracycline induction takes her close to

that ceiling. Deciding about this trial is therefore something that has to happen before or during induction, not after it.

Next steps

- 1. Email marcelo.cypel@uhn.ca and Jennifer.Lister@uhn.ca now and ask two things: does the study accept US-resident patients, and how is the 450 mg doxorubicin exclusion counted against a planned six-cycle induction.
- 2. Get the thoracic surgical read on resectability in parallel; the perfusion is coupled to metastasectomy and if she is not resectable the doxorubicin question is moot.
- 3. If the answer is that induction would disqualify her, that becomes a genuine input into the induction decision rather than a footnote.
- 4. Document the ECOG; the ceiling here is 2.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02811523		recruiting	unconfirmed	Marcelo Cypel, M.D.; marcelo.cypel@uhn.ca ; 416-581-7773

Payer / coverage: A Canadian trial for a US-resident patient raises cross-border coverage and travel questions that the study team should be asked about directly, before any of the clinical questions.

Notes: Recorded as a sequencing conflict rather than a simple eligibility yes or no, per the case instruction. The cumulative-anthracycline exclusion is the mechanism by which starting LMS-04 could permanently close this door.

46. Ivonescimab (PD-1 x VEGF bispecific)

Aliases: AK112, SMT112, ivonescimab-pd1-vegf

Modality: bispecific_antibody · **Regulatory:** Approved in China (NMPA) for non-small cell lung cancer indications. No FDA or EMA approval; investigational in the United States. · **Guidelines:** No guideline recognition in sarcoma · **Geographic scope:** United States, Memorial Sloan Kettering network only (New York and New Jersey) · **Verified:** 2026-08-24

This is the one strong-fit first-line seat the screen found. The MSK protocol nominally asks for one to three prior lines, but the criteria carry an explicit carve-out: patients who decline standard first-line systemic therapy are eligible, and the performance-status ceiling is ECOG 0-2 rather than 0-1. That makes it reachable now, before LMS-04, and it is the only investigational option in the dossier where declining induction is written into the protocol rather than argued around it. The cost is geography: all seven sites are Memorial Sloan Kettering locations in New York and New Jersey, and she lives at roughly 8,500 ft on the other side of the country.

Next steps

- 1. Call Dr D'Angelo's team at 646-888-4159 or email zzPDL_MED_Sarcoma_Clinical_Trials@mskcc.org this week and state plainly that she is systemic-therapy naive with LMS-04 planned, and ask whether the decline-first-line pathway is currently open and what documentation of the decline the protocol requires.

2. Ask in the same call which of the six satellite sites can perform infusions and which visits must happen in Manhattan; several sites are marked 'Limited Protocol Activities'.
3. Request release of the 2024 archival block now. The protocol requires sufficient archival tissue and the block sits at an outside institution, which is the slowest step in this whole dossier.
4. Document the ECOG. This protocol's ceiling of 2 is more forgiving than most, but an assumed score is still not a score.
5. Do not start pazopanib or any checkpoint inhibitor before this conversation concludes; both are exclusionary.
6. Work out the travel plan before committing: repeated flights from 8,500 ft to New York is a real burden and it should be priced honestly against the alternative of starting induction at home.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07526925		recruiting	likely	Sandra D'Angelo, MD; 646-888-4159

Manufacturer / sponsor contact

- **Company:** Summit Therapeutics Inc. (US and other rights); originator Akeso, Inc.
- **Med info email:** medinfo@smmmtx.com
- **Notes:** Email published on Summit Therapeutics' contact page for medical information and patient inquiries. Trial conduct is sponsored by Memorial Sloan Kettering, so screening questions go to the MSK contacts rather than to Summit.

Payer / coverage: Study drug is supplied. What is not supplied is travel: repeated cross-country trips for infusions and response assessment, plus whatever the protocol requires on-site rather than locally. Ask the study team at first contact which visits can be done at her home institution and whether MSK's patient travel assistance applies.

Notes: Reachable before LMS-04 starts, and the only investigational row in this dossier where that is true by explicit protocol language rather than by inference.

47. Letetresgene autoleucel (lete-cel, NY-ESO-1 TCR-T)

Aliases: lete-cel, GSK3377794, NY-ESO-1c259, IGNYTE-ESO

Modality: cell_therapy · **Regulatory:** Investigational; no approval anywhere. · **Geographic scope:** US, Canada, Europe; closed to new enrolment · **Verified:** 2026-08-24

The IGNYTE-ESO master protocol is closed to accrual, and it screens for NY-ESO-1 or LAGE-1a expression through a designated central laboratory. Her A*02:01 fits, but NY-ESO-1 has never been stained on her tissue and the study is not taking patients. Do not order NY-ESO-1 IHC on its own; it only makes sense as part of the shared cancer/testis block cut.

Next steps

1. Ask USWM at 1-855-246-9232 whether any IGNYTE-ESO cohort is reopening.

2. Order NY-ESO-1 IHC only inside tissue order C alongside MAGE-A4 and PRAME. Alone it is a wasted block cut.
3. The same $\geq 90\%$ room-air saturation and ECOG 0-1 constraints as the rest of the cell-therapy class should be assumed until a sponsor says otherwise.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03907223		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** USWM CT, LLC (US WorldMeds); developed by Adaptimmune Therapeutics
- **Med info phone:** 1-855-246-9232
- **Notes:** Phone from the TECELRA label; USWM is the registered sponsor for this protocol.

48. MASCT-I (autologous dendritic cell and cytokine-induced killer cell therapy) + doxorubicin and ifosfamide

Aliases: MASCT-I, DC-CIK, HRYZ

Modality: cell_therapy · **Regulatory:** Investigational; no approval outside the study. · **Geographic scope:** Mainland China only · **Verified:** 2026-08-24

A first-line study, which is rare in this dossier, but it runs at a single site in China and the cell product is manufactured there. Geography closes it. Recorded because a reader scanning for first-line trials will land on it and should see why it does not work.

Next steps

1. Confirm no US or EU site exists before considering further; as registered there is one site and it is in mainland China.
2. No realistic path for a US-based patient.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06227154		recruiting	no	Wenjin Huang; huangwenjin@shhryz.com; 021-37690016

49. Peposertib (DNA-PK inhibitor) + low-dose pegylated liposomal doxorubicin

Aliases: peposertib, M3814, MSC2490484A, nedisertib, pegylated liposomal doxorubicin, Doxil

Modality: small_molecule · **Regulatory:** Pegosertib investigational; no approval anywhere. Pegylated liposomal doxorubicin FDA-approved in other indications. · **Geographic scope:** United States; 16 sites including Aurora CO, Los Angeles, Chicago, Baltimore, Miami · **Verified:** 2026-08-24

A recruiting NCI study at 16 US sites, leiomyosarcoma named in the eligible histologies, ECOG ceiling 2. It requires at least one prior line, so it opens after LMS-04, but the criterion that has to be known before cycle six rather than after: prior anthracycline is permitted only if the cumulative dose before enrolment stays at or under 360 mg/m², and six full LMS-04 cycles at 60 mg/m² land exactly on that number. Whether this seat survives induction is decided by cycle count and dose reductions, which makes it a sequencing constraint on the induction plan, not a later-line footnote.

Next steps

- 1. Put the 360 mg/m² arithmetic in front of the treating oncologist before induction starts, not before cycle six: six full cycles at 60 mg/m² equals exactly 360, so either a five-cycle plan, dose reductions, or a decision to forgo this trial should be explicit up front.
- 2. Ask the team to track cumulative anthracycline dose in the chart in mg/m² as induction proceeds, so the enrolment ceiling is a number on file rather than a reconstruction.
- 3. Call the Colorado site at 720-848-0650 after the first line to confirm slot availability and how the site counts cumulative dose (dosed versus received).
- 4. Confirm a lung lesion is biopsy-amenable; the protocol biopsy is mandatory and her infiltrative pattern needs an interventional-radiology read.
- 5. Document the ECOG; the ceiling of 2 is workable but the score must exist.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05711615		recruiting	no	—

Payer / coverage: Study drugs supplied on trial. The real costs are the mandatory biopsy and travel; the Aurora, Colorado site is the practical anchor for a mountain-state patient.

Notes: Same shape as the Toronto in vivo lung perfusion conflict: an anthracycline ceiling that LMS-04 induction can consume. The perfusion trial caps at 450 mg total and this one at 360 mg/m² cumulative; both belong in the induction-planning conversation in the first-line window even though neither is enrollable until after it.

50. Pulmonary suffusion with cisplatin plus metastasectomy

Aliases: pulmonary suffusion, cisplatin, minimal residual disease

Modality: surgery · **Regulatory:** Cisplatin is approved; the suffusion delivery is investigational. · **Geographic scope:** United States; single site, Buffalo NY · **Verified:** 2026-08-24

Recruiting at Roswell Park, sarcoma is in scope, ECOG ceiling is 2, and there is no prior-line requirement in the registry criteria. What it does require is that a surgeon judges her pulmonary function sufficient for the planned metastasectomy, which puts the same resectability question in front of her that the Toronto study does. Unlike that

study it is in the US.

Next steps

1. Email Kenneth.Seastedt@Roswellpark.org with the current chest imaging and ask whether her bilateral infiltrative pattern is within the resectable population this study treats.
2. Have the PFTs with DLCO done at her home altitude before that conversation; the surgeon's judgement of pulmonary reserve is the gate and an altitude-naive reading will mislead.
3. Note the 4-week washout from chemotherapy, which matters for timing around induction cycles.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03961234		recruiting	unconfirmed	Kenneth Seastedt; Kenneth.Seastedt@Roswellpark.org; 716-845-2300

51. REC-617 (selective CDK7 inhibitor)

Aliases: REC-617, CDK7 inhibitor

Modality: small_molecule · **Regulatory:** Investigational; no approval anywhere. · **Geographic scope:** United States; single site, Houston TX · **Verified:** 2026-08-24

Recruiting at MD Anderson for leiomyosarcoma of any primary site, but it needs two things she does not have: an RB1-negative result by IHC at a 0% cutoff, and at least one prior line of systemic therapy. The ECOG ceiling is 2, which is unusually forgiving. RB1 status is currently unmeasured, so ordering Rb by IHC in tissue order B keeps this option alive without any extra tissue.

Next steps

1. Include Rb by IHC in tissue order B so the eligibility question is already answered when the time comes.
2. Email efnassif@mdanderson.org after the first line to confirm slot availability and whether the site's own IHC is required.
3. Not accessible before induction; the prior-line criterion is explicit.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07631756		recruiting	no	Elise Nassif, MD; efnassif@mdanderson.org; 281-460-0607

52. Tuvusertib (ATR inhibitor) + temozolomide

Aliases: tuvusertib, M1774, temozolomide, Temodar

Modality: small_molecule · **Regulatory:** Tuvusertib investigational; no approval anywhere. Temozolomide FDA-approved in glioma, generic. · **Geographic scope:** United States; 23 sites · **Verified:** 2026-08-24

The one live US ATR seat. The NCI dose-escalation arm admits extracranial solid tumours where temozolomide is guideline-standard, which covers soft-tissue sarcoma without requiring an MGMT result, and the ECOG ceiling is 2. It requires progression on, intolerance of, or refusal of available therapies, so in practice it opens after LMS-04. Two sequencing traps sit upstream: prior temozolomide is disqualifying, so an off-label olaparib plus temozolomide line would close this door, and the ATR-for-ALT rationale should be priced as unproven in her histology, because the ATRX-mutant leiomyosarcoma monotherapy cohort progressed within 4 months despite biopsy-proven target engagement.

Next steps

1. After LMS-04, call the nearest recruiting site (UC Irvine 877-827-8839, Yale 203-785-5702, or Kansas 913-588-3671) and confirm the dose-escalation arm is still open to sarcoma; the phase 2 portion is colorectal-only and the arm structure decides whether a seat exists.
2. Keep temozolomide out of any off-label interim plan if this seat is being held; prior temozolomide is disqualifying.
3. Include ATRX in tissue order A regardless. It does not gate this trial's entry, but it determines whether the ALT rationale for choosing this seat over other 2L options has any footing.
4. Document the ECOG; ceiling 2.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05691291	2	recruiting	no	—

Payer / coverage: Study drug supplied on trial; temozolomide on protocol is not the payer problem it is off-label.

Notes: Do not price this seat on the ALT/ATRX thesis: that thesis was tested in her histology as monotherapy and failed (progression inside 4 months with confirmed target engagement). The honest rationale for this trial is the temozolomide combination, not the ATRX biology.

53. Ubamatamab (MUC16 x CD3 bispecific), alone or with cemiplimab

Aliases: REGN4018, MUC16xCD3

Modality: bispecific_antibody · **Regulatory:** Investigational; no approval. · **Geographic scope:** United States; single site, closed to accrual · **Verified:** 2026-08-24

This MD Anderson study is limited to renal medullary carcinoma and epithelioid sarcoma with confirmed loss of SMARCB1 staining, and it is closed to accrual. Her histology is outside it twice over. Recorded so the SMARCB1 branch shows every door that was checked.

Next steps

1. No action. Histology excludes her independently of the enrolment status.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06424880		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Regeneron Pharmaceuticals, Inc.
- **Med info phone:** 1-877-542-8296
- **Notes:** Phone from the current Libtayo (cemiplimab) FDA label; Regeneron's general medical information line. No ubamatamab-specific line is published.

54. Uzatresgene autoleucel (ADP-A2M4CD8), alone or with nivolumab or pembrolizumab

Aliases: ADP-A2M4CD8, SURPASS, uzatresgene

Modality: cell_therapy · **Regulatory:** Investigational; no approval anywhere. · **Geographic scope:** US, Canada, Spain; closed to new enrolment · **Verified:** 2026-08-24

SURPASS is the MAGE-A4 TCR-T study that admits tumour types beyond synovial sarcoma, and it is closed to accrual. It also carries the criterion that recurs across this whole class and matters most in her case: at least 90% room-air oxygen saturation at rest, measured at screening and again at baseline. At 8,500 ft a resting room-air saturation in the high 80s can be physiologically normal, and the cell-therapy protocols in this class write the criterion as an absolute number with no altitude clause. Get the oximetry done and recorded with the altitude noted, and raise the interpretation with the sponsor's medical monitor before screening rather than after a failed check.

Next steps

1. Confirm with USWM at 1-855-246-9232 whether any SURPASS cohort is reopening; the registry status is 'active, not recruiting' rather than terminated.
2. Get resting and exertional oximetry at her home altitude and record the altitude with the value, so the $\geq 90\%$ criterion can be discussed with a medical monitor on facts rather than on a sea-level reading taken during a specialist visit.
3. Document the ECOG; this protocol caps at 0-1.
4. Order MAGE-A4 IHC as part of tissue order C only if a MAGE-A4 program is actually open to her.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04044859		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** USWM CT, LLC (US WorldMeds); developed by Adaptimmune Therapeutics
- **Med info phone:** 1-855-246-9232
- **Notes:** Phone from the TECELRA label; USWM is the registered sponsor for the SURPASS protocol.

Notes: The 90% room-air saturation criterion is a class-wide feature, not a quirk of this protocol. It applies to the cell-therapy programs generally and is the single criterion most likely to be misread in a patient adapted to 8,500 ft.

55. Valemetostat tosylate (EZH1/EZH2 dual inhibitor)

Aliases: DS-3201, Ezharmia, valemetostat

Modality: small_molecule · **Regulatory:** Approved in Japan (Ezharmia) for adult T-cell leukemia/lymphoma and peripheral T-cell lymphoma. No FDA or EMA approval; investigational in solid tumours. · **Geographic scope:** France only as registered; no US site · **Verified:** 2026-08-24

The trial route for the SWI/SNF branch is a Gustave Roussy basket study, and it is stricter than the tazemetostat data: it requires a documented bi-allelic homozygous deletion of SMARCB1 by validated NGS plus centrally confirmed loss of expression by IHC. A single plasma frameshift does not meet that. It is also France-only, four sites, with an ECOG 0-1 ceiling and a maximum of one prior treatment line in the SMARCB1 cohort.

Next steps

1. Confirm a US or EU site is accepting international patients before anything else. As registered this is a France-only study and she is US-based.
2. Email Dr Postel-Vinay at SOPHIE.POSTEL-VINAY@gustaveroussy.fr to ask whether a truncating SMARCB1 frameshift with confirmed INI1 loss by IHC can substitute for a documented homozygous deletion. That single answer decides whether the trial is even theoretically open to her.
3. Order the tissue NGS and INI1 IHC now; both are prerequisites for any version of this conversation.
4. Note the one-prior-line cap: after LMS-04 induction she would still be within it, but only just.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07323387		recruiting	no	Sophie Postel-Vinay, MD, PhD; +33142114343

Payer / coverage: Not a payer question. Study drug is supplied on trial; the cost is travel and the screening biopsy work.

Notes: A reader scanning the trial table sees a recruiting SMARCB1 study and assumes a door. The bi-allelic homozygous deletion requirement and the France-only geography are both hard, and her plasma frameshift satisfies neither.

56. ZL-6201 (LRRC15-directed antibody-drug conjugate)

Aliases: ZL-6201, LRRC15

Modality: antibody_drug_conjugate · **Regulatory:** Investigational; no approval anywhere. · **Geographic scope:** United States and China · **Verified:** 2026-08-24

Recruiting with US and China sites, and sarcoma patients may have received up to two prior lines of systemic therapy in the metastatic setting. ECOG ceiling is 1. Like the other first-in-human ADCs here, it is a post-induction option with a line ceiling that eventually closes it.

Next steps

1. Email ZL-6201-001@zailaboratory.com to identify which of the US sites are open and whether LRRC15 expression is required or only collected.
2. Post-induction option; the two-line ceiling gives a window rather than an open door.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07374848		recruiting	no	ZL-6201-001 Study Team; ZL-6201-001@zailaboratory.com ; 650.360.1601

Manufacturer / sponsor contact

- **Company:** Zai Lab (Shanghai) Co., Ltd.
- **Med info phone:** 650.360.1601
- **Med info email:** ZL-6201-001@zailaboratory.com
- **Notes:** Contact captured verbatim from the [NCT07374848](#) registry record.

57. Zanzalintinib (XL092)

Aliases: XL092, zanzalintinib

Modality: small_molecule · **Regulatory:** Investigational; no approval anywhere. · **Geographic scope:** United States; single site, Chicago IL · **Verified:** 2026-08-24

Recruiting at a single US site, Northwestern, and its leiomyosarcoma cohort requires at least two prior lines of antineoplastic treatment. She has none. This is a third-line conversation at the earliest, and it also requires pathology review at an NCCN-designated centre, which is worth arranging early since her original specimen was signed out benign elsewhere.

Next steps

1. Not accessible in the first-line window. Park until at least two lines are behind her.
2. Arrange NCCN-designated-centre pathology review as part of the reference-laboratory work already underway; it satisfies this protocol's requirement and several others.
3. Recheck cohort status at second progression via cancer@northwestern.edu.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06521734		recruiting	no	Coordinator; cancer@northwestern.edu; 3126951301

Not yet accessible (1)

58. Tuvusertib monotherapy in ATRX-mutant astrocytoma (GEINO, Spain)

Aliases: tuvusertib, M1774, GEINO

Modality: small_molecule · **Regulatory:** Investigational; no approval anywhere. · **Geographic scope:** Spain only; astrocytoma only · **Verified:** 2026-08-24

Not enrollable for her on two hard grounds: astrocytoma-only histology and Spanish sites. It earns a row anyway because it is the only registered trial anywhere that uses ATRX mutation as the entry gate for an ATR inhibitor, which is exactly the selection logic the preclinical file proposes for her tumour type. If her tissue NGS returns ATRX loss, this study's read-out becomes the nearest future test of whether that selection logic works in anyone.

Next steps

1. No enrolment action. Include ATRX in tissue order A so the relevance of this read-out to her case is decidable.
2. Flag for the next evidence re-screen: if this study reports and her tumour carries ATRX loss, the ATR branch should be re-priced then.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07427761		recruiting	no	GEINO Secretary; secretaria@geino.es; 0034934344412

Unavailable (6)

59. BRD9 degraders (class)

Aliases: FHD-609, CFT8634, ncBAF, dBRD9

Modality: targeted_protein_degrader · **Regulatory:** No approval anywhere. Both clinical programs terminated. · **Geographic scope:** No open access anywhere · **Verified:** 2026-08-24

The two clinical BRD9 degraders that would have been the second SMARCB1-directed option are both terminated. Foghorn's FHD-609 and C4 Therapeutics' CFT8634 each ran phase 1 studies in synovial sarcoma and SMARCB1-null tumours and both closed without a successor program on the registry. There is no trial, no expanded access and no approved drug in this class.

Next steps

- 1. No action available. Recorded so the branch shows as closed rather than missing.
- 2. If a next-generation BRD9 degrader enters the clinic, the SMARCB1 confirmation work done now would carry over.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04905753		terminated	no	—
NCT05345753		terminated	no	—

60. Berzosertib + gemcitabine (randomised phase 2, Institut Bergonié)

Aliases: berzosertib, M6620, VX-970, gemcitabine

Modality: small_molecule · **Regulatory:** Berzosertib investigational; no approval anywhere. · **Geographic scope:** France only; completed · **Verified:** 2026-08-24

A read-out to watch, not a seat. The randomised leiomyosarcoma-specific test of gemcitabine with or without ATR inhibition completed accrual in France and no result is indexed as of August 2026. There is nothing to enrol in and nothing to prescribe; its value is that it will say whether ATR inhibition adds anything to chemotherapy in her histology after the monotherapy cohort failed.

Next steps

- 1. No access action. Ask whoever re-runs the evidence screen to check for the publication at the next progression decision point; a positive read-out would change how the tuvusertib seat is weighed.
- 2. Nothing about this row should delay or alter present treatment.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04827816		completed	no	—

Notes: Classified unavailable rather than clinical_trial_only because there is no enrolment path and no drug supply route; the row exists so the pending read-out is visible in the access guide.

61. Berzosertib monotherapy (ATRX-mutant leiomyosarcoma cohort, NCT03718091)

Aliases: berzosertib, M6620, VX-970, VE-822, atr-inhibition-alt

Modality: small_molecule · **Regulatory:** Investigational; no approval anywhere. No active berzosertib monotherapy program in sarcoma. · **Geographic scope:** No open access anywhere · **Verified:** 2026-08-24

Closed, and the result is the point: the ATRX-mutant leiomyosarcoma cohort, built on the same ALT rationale the preclinical file carries, progressed within 4 months on ATR-inhibitor monotherapy despite biopsy-proven target engagement. There is no berzosertib program to join and no access request to make. This row exists so the access guide carries the negative alongside the hypothesis.

Next steps

1. No action. Carry the negative into any future weighing of ATR seats: monotherapy in her histology has been tried and did not hold.
2. If ATR inhibition returns to the table it should be as a combination, which is what the tuvusertib plus temozolomide row offers.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03718091		completed	no	—

Notes: Supersedes the hedged framing in the earlier atr-inhibition-alt row (August 2026), which recorded only a failed in-vitro replication: the clinical monotherapy test in ATRX-mutant leiomyosarcoma has also read out negative, so ATR seats should not be priced as though the ALT/ATRX biology were established for her.

62. CFT8634 (BRD9 degrader)

Aliases: CFT8634

Modality: targeted_protein_degrader · **Regulatory:** No approval; phase 1 terminated. · **Geographic scope:** No open access anywhere · **Verified:** 2026-08-24

C4 Therapeutics stopped this phase 1 in SMARCB1-perturbed cancers and no follow-on program is registered. As with FHD-609, the class is closed for now.

Next steps

1. No action available.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05335753		terminated	no	—

Manufacturer / sponsor contact

- **Company:** C4 Therapeutics, Inc.
- **Notes:** No medical information line for the terminated program was verifiable this run.

63. FHD-609 (BRD9 degrader)

Aliases: FHD-609

Modality: targeted_protein_degrader · **Regulatory:** No approval; phase 1 terminated. · **Geographic scope:** No open access anywhere · **Verified:** 2026-08-24

Foghorn's phase 1 in synovial sarcoma and SMARCB1-loss tumours is terminated on the registry, with no successor study listed. Nothing to enrol in and nothing to request.

Next steps

1. No action available.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04905753		terminated	no	—

Manufacturer / sponsor contact

- **Company:** Foghorn Therapeutics Inc.
- **Notes:** No medical information line published for a terminated investigational program was verifiable this run.

64. [177Lu]Lu-ITG-PSMA-1 with [68Ga]Ga-PSMA-11 PET selection

Aliases: ITG-PSMA-1, PSMA-11, vascular disruption

Modality: radioligand · **Regulatory:** Investigational; the sarcoma application has no approval and no active program. · **Geographic scope:** No open access anywhere · **Verified:** 2026-08-24

A completed single-centre phase 1 in Switzerland testing a vascular-disruption approach in soft tissue sarcoma. It is finished, it was never open outside Lausanne, and no successor study is registered. There is no route to this.

Next steps

1. No action available.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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NCT05420727

completed

no

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